Introduction 1

LEARNING OBJECTIVES

1.1 Explain how ethical, legal, and professional behaviors are different from each other.

1.2 Articulate the process involved in ethical decision making.

1.3 Describe how to recognize legal issues and how to obtain legal advice.

FOCUS QUESTIONS

1. Assuming that you are a moral and responsible person (as are most counselors), why do you think it is important for you to study ethical and legal principles and the decision-making process?
2. What are the differences among legal, ethical, and professional behaviors?
3. What resources can you use when you need help in resolving an ethical dilemma?
4. How should you get legal advice when a legal issue presents itself?

This text is intended primarily for prospective counselors, so most readers are likely to be graduate students in counselor education programs. However, many counselors who are already practicing use this text as a resource to help them address legal and ethical issues. Whether you are a student or a practicing professional counselor, we hope you will develop a thoughtful understanding of ethical, legal, and professional issues in counseling as you digest and discuss the material.

Before you begin exploring these issues, it may be helpful to have an overview of how this text is organized. Section I contains this introductory chapter and two additional chapters that address professional identity and diversity. These are cross-cutting issues that permeate every ethical, legal, and professional issue that is discussed in Section II. Each chapter in Section II focuses on a specific issue such as confidentiality, competence, boundaries of the counseling relationship, and technology in counseling.

Since the previous edition of this text was created, the entire world experienced upheaval caused by the COVID-19 pandemic. The pandemic not only disrupted lives but also caused profound changes in the ways that counseling is taught and practiced. Virtual counseling, supervision, and education are now the norm, even in our post-pandemic environment. Thus, each chapter now contains a short, new section that describes the impact of the COVID-19 pandemic on the issue(s) addressed in that chapter.

The Impact of the COVID-19 Pandemic on the Counseling Profession

During the early months of 2020, as the severity of the COVID-19 pandemic became apparent, U.S. society went into lockdown. The widespread upheaval in people's lives included dealing with a life-threatening illness, the deaths of loved ones, loss of employment, widespread fear and anxiety, and profound disruptions of daily routines of living. The impact on the counseling profession was dramatic, as counseling and counselor education were forced to switch immediately to virtual means of delivery. Adaptations and transitions occurred at a remarkably fast pace so that counseling services could continue to be available, now in a virtual format, to most clients. Counselor education programs switched to online delivery as well, and students were able to continue with their studies and to graduate on time. These changes, born of necessity, turned out to have many advantages, and virtual counseling and virtual counselor education have continued into the post-pandemic era, taking their place alongside the resumption of face-to-face counseling and education. It will take time to fully understand the impact of all these changes, but at this point it is evident that no aspect of the counseling profession, including ethical practice and legal requirements, remains untouched by the pandemic. The ethical, legal, and professional issues raised by this transformation are discussed in more detail in ensuing chapters.

PROFESSIONAL ORIENTATION

Ethical, legal, and professional issues, collectively, make up the *professional orientation* content area of your graduate studies. A fundamental part of your professional development as a counselor is to acquire a firm grounding in the area of professional orientation. This content area includes three main components:

- ***Developing a professional identity as a counselor.*** This includes understanding the history and development of counseling and related professions, knowing the professional roles and functions of counselors and how these roles and functions are similar to and different from those of other professions, learning about and becoming involved in professional organizations, gaining awareness of counselor preparation standards and credentialing, knowing how to advocate for your clients and your profession, and developing pride in your profession. Professional identity is discussed in detail in Chapter 2.
- ***Learning about ethics.*** This involves becoming familiar with ethical standards for counselors, understanding the ethical issues that counselors encounter, developing ethical reasoning and decision-making skills, and being able to use an ethical decision-making model to apply your knowledge and skills in your day-to-day professional activities.
- ***Learning about the law as it applies to counseling.*** This includes being able to distinguish among legal, ethical, and clinical issues; acquiring a basic knowledge of legal issues in counseling and the laws that affect the practice of counseling; and knowing what to do when you are faced with a legal problem.

The importance of professional orientation is evident in standards for preparation and credentialing of counselors. The Council for Accreditation of Counseling and Related Educational Programs (CACREP) sets standards for counselor preparation and accredits training programs that meet these standards. A primary purpose of the CACREP Standards (2024) is to ensure that students graduate with a strong professional identity. CACREP requires the curriculum for counselors-in-training

to include studies that provide an understanding of professional functioning. Further information about CACREP is provided in Chapter 2.

The National Board for Certified Counselors (NBCC), a voluntary organization that credentials counselors, also requires the counselors it certifies to complete coursework in the area of professional orientation to counseling (NBCC, 2024). If you plan to become licensed as a professional counselor, you should be aware that state counselor licensure boards mandate that licensees demonstrate knowledge of professional orientation issues, which include ethical and legal issues.

Beyond external requirements, it is essential that you develop a strong professional identity as a counselor during this time, when our profession is facing both new and ongoing challenges from other mental health professions. Counseling is a relatively young profession, and the general public may not be as aware of professional counselors' identity as they are aware of the identities of psychologists, social workers, and psychiatrists. You may be asked, "What kind of counselor are you?" or "Is being a counselor like being a psychologist?" or "How are counselors different from social workers?" These are legitimate questions, and you must be prepared to clearly explain who you are as a member of this professional group, what you believe, how you are similar to other mental health professionals, and, more importantly, how you are different from those professionals. You must also be prepared to practice in ways that are ethically and legally sound and that promote the welfare of your clients. Information throughout this text will provide you with an understanding of your chosen profession of counseling and will prepare you to practice in an ethical and legal manner.

We hope that seasoned practitioners, as well as counselors-in-training, will read this text and find it useful. Professional, ethical, and legal standards are constantly changing, and it is important to keep up to date. Also, as Corey et al. (2024) have pointed out, issues that students and beginning practitioners encounter resurface and take on new meanings at different stages of one's professional development.

In the remainder of this section, we clarify some important terms, distinguishing among *morals, values*, and *ethics* as these terms are used in this text. We also delineate the differences and areas of overlap among *ethical, legal*, and *professional* issues as they apply to the practice of counseling. Finally, we offer a model of professional practice that demonstrates how the multiple components of practice are related to and connected with each other.

Morals, Values, and Ethics

The terms *morals, values*, and *ethics* are sometimes used interchangeably, and they do have overlapping meanings. For instance, all three terms involve judgments about what is good and bad, or right and wrong, and all pertain to the study of human conduct and relationships. Nonetheless, distinctions must be drawn when these terms are applied to the behaviors of professional counselors so that a shared understanding can be established.

The term *moral* is derived from the Latin word *mores*, which means customs or norms. Moral actions are determined within a broad cultural context or religious standard. Although some moral principles, such as "Do no harm to others," are shared by most civilized groups of people, how these moral principles are interpreted and acted on will vary from culture to culture and from individual to individual within a culture. Thus, conduct that you evaluate as moral might be judged as immoral by another person or by people in another society. It is important to remember that what you view as moral behavior is based on your values. In this text, when we refer to moral conduct, we ask you to think in terms of your *personal* belief system and how this belief system affects your interactions with others in all aspects of your life.

Although values are very similar to morals in that they serve as a guide to determining what is good or right behavior, we use the term *values* to apply broadly to both the personal and the professional functioning of counselors. Our *personal* values guide our choices and behaviors, and each of us holds some values more strongly than other values. Your value system is unique to you, and it has been influenced by your upbringing, the culture in which you live, and quite possibly your religious beliefs. What is important about your personal values as they relate to professional practice is that you have a high level of self-awareness of your values and that you learn to *bracket* (Kocet & Herlihy, 2014), or set aside, your personal values within the counseling relationship. One of the hardest lessons counselors must learn is to respect values that are different from their own and to avoid imposing their own personal values on their clients. It is particularly difficult to avoid imposing your values when an implicit bias (a bias of which you are not aware) is at work or when a client holds values that are very different from your own (Thacker & Blueford, 2018). For example, if you believe strongly that women have the right to choose whether to terminate a pregnancy, then it will be challenging for you to keep your values in check as you counsel a woman who expresses her support for a total ban on abortion. Similarly, it may be difficult for you to counsel clients who are seeking divorce if you believe strongly in the sanctity of marriage. A series of court cases have involved counselors with strong religious beliefs who declined to provide affirmative counseling to clients who desired to improve their same-sex romantic relationships. Partly because of the controversy generated by these court cases, the *Code of Ethics* of the American Counseling Association (2014) includes several standards that make it quite clear that counselors must not impose their own personal values on their clients.

Members of the counseling profession share certain *professional* values. These include enhancing human development across the life span, honoring diversity and embracing a multicultural approach, promoting social justice, safeguarding the integrity of the counselor–client relationship, and practicing competently and ethically (ACA, 2014, *Code of Ethics*, Preamble). These core values are articulated in the *Code of Ethics* to help acculturate students to the expectations of the counseling profession (Francis, 2015). If a counseling student's personal values were so strong that he or she could not learn to counsel clients who held differing beliefs, or if a student could not embrace the professional values of the profession as articulated in the *Code of Ethics*, we would be concerned that the student is not well suited for the counseling profession.

1-1 The Case of Carole

Carole is a master's-degree student in counseling who is enrolled in her practicum course and has begun to work under supervision with her first clients. Carole self-identifies as mixed-race Latine: Her father is White, and her mother emigrated from a country in Central America. Carole grew up hearing stories of the oppression and violence that her mother had experienced in her home country before coming to the United States, and she has been saddened and angered to see the prejudice that her mother continues to encounter as a member of the Latine community.

Now in her sixth week of practicum, Carole has received positive feedback about her work with clients. Today, however, she had a new client named Frank, who sought counseling for depression that he believes is caused by his inability to find a job. During the session, Frank spoke at length and with bitterness about immigrants from Mexico and Central American countries who "have taken all the jobs that rightfully belong to real Americans." He also used a number of racial slurs as he was speaking. Carole found herself feeling angry at Frank, and she struggled to get through the session.

During her feedback session regarding her work with the client, Carole acknowledged that she had performed poorly in the counseling session and insisted that she would never be able to work with clients like Frank who are prejudiced against people who share her heritage. She asked to be reassigned to a different client.

- What do you think of Carole's request to be reassigned so that she will not have to continue working with Frank?
- If you were Carole's supervisor, what would you tell her?

Discussion: Although Carole's emotional reaction to the client's prejudice toward people who share her heritage is understandable, she must learn to bracket, or set aside, her personal values during her counseling sessions. Her supervisor will work with her to help her process her reaction and learn to effectively counsel clients who "push her personal buttons." Actually, Carole is fortunate to have encountered a client like Frank while she is in her practicum and can receive the assistance she needs to become competent to work with such clients. Her supervisor will make it clear that referring Frank to a different counselor is not the solution to her difficulties and that Carole is expected to develop the competence to work with clients who evoke uncomfortable personal reactions.

Ethics is a discipline within philosophy that is concerned with human conduct and moral decision making. Certainly, you have developed your own individual ethical stance that guides you in the ways you treat others, expect them to treat you, and make decisions about what behaviors are good or right for you. In this text, however, we think of ethics as it relates to the profession of counseling; that is, ethics refers to conduct judged as good or right for counselors as a professional group. When your fellow professionals have come to sufficient consensus about right behaviors, these behaviors have been codified and have become the ethical standards to which you are expected to adhere in your professional life (ACA, 2014). Therefore, think about ethics as referring to your *professional* behavior and interactions. Keep in mind that ethics must prevail over your personal values when value conflicts arise within a counseling relationship. Because the counseling relationship exists to benefit the client, you must avoid imposing your own values on your clients.

Legal, Ethical, and Professional Behavior

Law is different from morality or ethics even though law, like morality, is created by a society, and like ethics, it is codified. Laws are the agreed-upon rules of a society that set forth the basic principles for living together as a group. Laws can be general or specific regarding both what is required and what is allowed of individuals who form a governmental entity. Criminal laws hold individuals accountable for violating principles of coexistence and are enforced by the government. Civil laws allow members of society to enforce rules of living with each other.

Our view is that there are few conflicts between law and ethics in professional counseling. Keep in mind, though, that there are important differences. Laws are created by elected officials, enforced by police, and interpreted by judges. Ethics are created by members of the counseling profession and are interpreted and enforced by ethics committees and licensure and certification boards. Laws dictate the *minimum* standards of behavior that society will tolerate, whereas ethics pertains to a wider range of professional functioning. Some ethical standards prescribe the minimum that other counselors will tolerate from fellow professionals (for example, sexual or romantic relationships with clients are prohibited), and some ethical standards describe ideal practices to which counselors should aspire (for example, counselors shouldaspire to foster meaningful and respectful professional relationships).

Rowley and MacDonald (2001) discussed the differences between law and ethics using concepts of culture and cross-culture. They argued that "law and ethics are based on different understandings of how the world operates" (p. 422). These authors advise you to learn the different culture of law, seek to understand how law operates, and develop collaborative partnerships with attorneys. We agree with the perspective that the cultures of counseling and law are different and that seeking legal advice is often an important step in the practice of counseling.

Where does the notion of *professionalism* fit into the picture? Many factors—including the relative newness of the counseling profession, compared to other mental health professions; the interpersonal nature and complexity of the counseling process; and the wide variety of types of counselors and their work settings—make it essential for counselors to conduct themselves in a professional manner. However, it is not easy to define what it means to be *professional*, and we discuss this in more detail in Chapter 2. Legal standards are the minimum that society will tolerate from a professional. Ethical standards occupy a middle ground, describing both the minimal behaviors expected of counselors and the ideal standards to which counselors should aspire. Although professionalism is related to ethics, it is possible to be unprofessional without being unethical. For instance, a counselor might frequently run a bit late for counseling sessions, which might be considered unprofessional but would not be unethical. By contrast, if a counselor were to enter into a sexually intimate relationship with a client, this behavior would be unethical, illegal, *and* unprofessional and would violate the professional values that counselors strive to uphold.

Professionalism is closely related to the concept in a profession of *best practice*, and perhaps the concepts of law, ethics, and best practice in the field of counseling are on a continuum. Best practice is the very best action a counselor could be expected to do. Best practice guidelines are intended to provide counselors with goals to which they can aspire, and they are motivational, as distinguished from ethical standards, some of which are aspirational while others are enforceable.

Although counselors vary to an extent in their beliefs about what constitutes best practice, you will want to strive to practice in the best possible manner and to provide the most competent services to your clients throughout your career. Meeting minimum legal standards or minimum ethical standards is not enough for the truly professional counselor. Rather, professionalism demands that you be the best counselor for your clients that you are capable of being.

1-2 The Case of Samya

Samya, who works in a community agency, will be seeing a 16-year-old minor for his first counseling session. Samya knows that legally and ethically, she must have one of his parents sign an agreement permitting her to disclose information regarding his sessions to his parent's health insurance company so that the parent will be reimbursed partially for the cost of her counseling services. Samya also is aware that, according to the ACA *Code of Ethics* (2014), she may include his parents in the counseling process, as appropriate (§A.2.d.; §B.5.b). However, she realizes how important confidentiality is to adolescents, and she wants to provide services to this minor in a way that would meet best practice standards.

- What are some of the things Samya might do in this situation to go beyond what is minimally required by law or the ACA *Code of Ethics* (2014)?
- How will Samya know if what she finally decides to do is best practice?

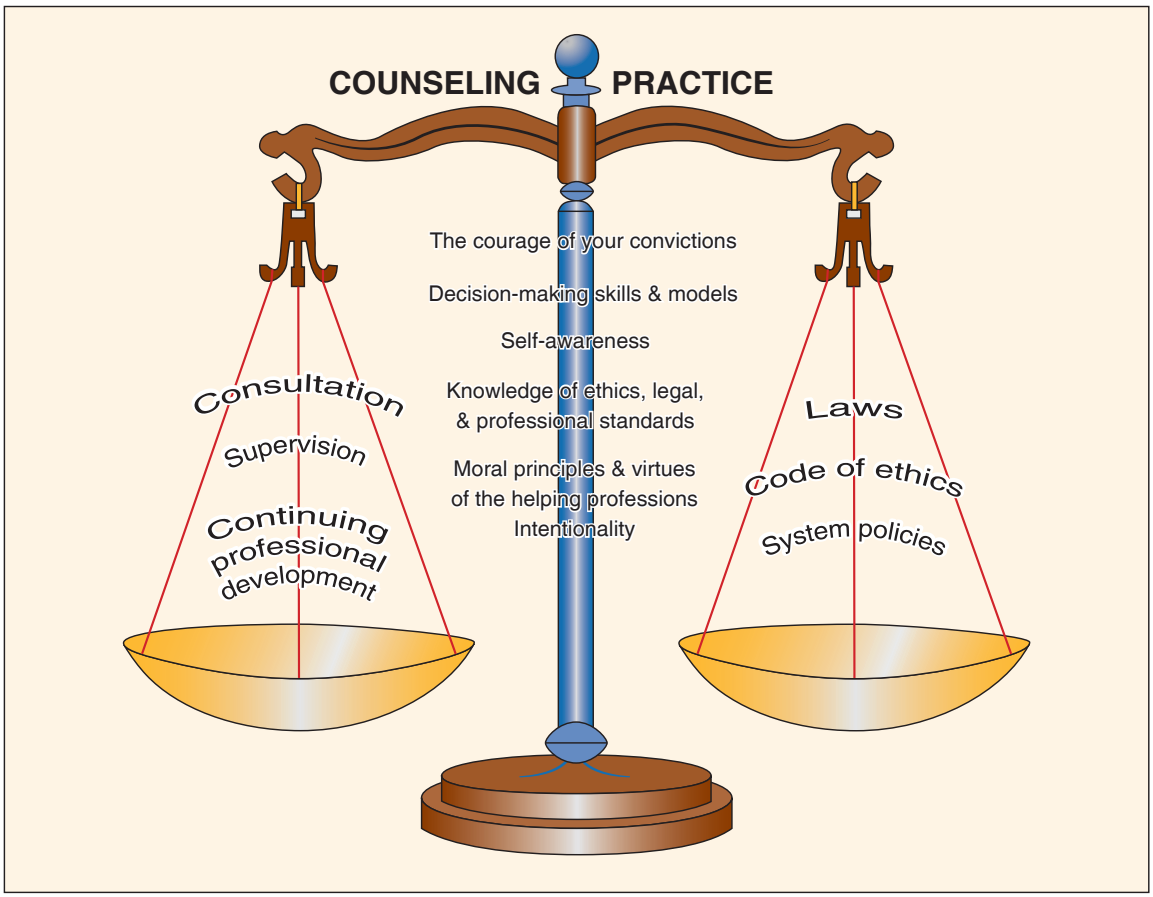
Discussion: You will have the information you need to answer these questions after you have read material on ethical decision making, informed consent, confidentiality, and counseling

minor clients, all presented later in the text. For now, a brief answer is that Samya would be well advised to hold a conversation with both the client and his parent(s) present, in which she discusses confidentiality and its limits (including the information she would share with the insurance company). Including the client in the decision-making process is good practice, and Samya can ask the client to sign the agreement to signify his assent, in addition to having the parents sign to give legal consent. Best practice for Samya will mean keeping a careful balance, one that honors both her minor client's right to privacy and his parents' rights to information about their son, and working to establish and maintain a cooperative relationship with all parties.

A Model for Professional Practice

One source of very real frustration for prospective and beginning counselors is that there are so few absolute, right answers to ethical, legal, or best practice questions. Throughout your career, you will encounter dilemmas for which there are no *cookbook* solutions or universally agreed-upon answers. We visualize professional practice as entailing a rather precarious balance that requires constant vigilance. We also see counseling practice as being built from within the self but balanced by outside forces, as shown in Figure 1-1.

FIGURE 1-1 Professional practice—built from within but balanced from outside the self



In this model of professional practice, the internal building blocks climb up the post that holds the arms of the scale. The most fundamental element, at the base, is *intentionality*. Being an effective practitioner must start with good intentions, or wanting to do the right thing. The overwhelming majority of counselors have the best intentions: They want to be helpful to those they serve.

The second building block contains *principles and virtues*. Principles and virtues represent two philosophies that provide the underpinnings of ethical reasoning. Moral principles are a set of shared beliefs or agreed-upon assumptions that guide the ethical thinking of helping professionals (including physicians, nurses, and other medical specialists; teachers; and mental health professionals). Basic moral principles include respect for autonomy (honoring freedom of choice), nonmaleficence (doing no harm), beneficence (being helpful), justice (being fair), fidelity (being faithful), and veracity (being honest). Virtue ethics focuses on the traits of character or dispositions that promote the human good. We discuss these in more detail later in this chapter.

The third element is *knowledge* of ethical, legal, and professional standards. You will find that a wealth of resources is available to you as you work to gain, maintain, and expand your knowledge base. Texts such as this one, casebooks, professional journals, codes of ethics, workshops and seminars, professional conferences, some internet sites, and your supervisors and colleagues are all excellent resources that can help to increase your knowledge.

The fourth element is *self-awareness*. As discussed earlier in this chapter, counselors must maintain a high level of self-awareness so that they do not inadvertently impose their own values, beliefs, and needs onto their clients. Knowledge of ethical, legal, and professional standards is not sufficient: Best practice is achieved through constant self-reflection and personal dedication, rather than through mandatory requirements of external organizations (Francis, 2015).

Even after you have developed a solid knowledge base and the habit of self-reflection, you must have *skills* for applying your knowledge and reasoning through the questions and dilemmas that will arise in your practice. You are also expected to have a *model* that will serve as a road map to guide your ethical decision making and bring some consistency to your decision-making process.

The final internal element is *the courage of your convictions*. This element can challenge even the most conscientious counselors who have the best intentions. As a counselor, you will face ethical quandaries. It can take courage to do what you believe is right, especially when there is a high cost to yourself, when your personal needs are involved, when you know that others may not agree with or approve of your actions, or when (as is the case in ethical dilemmas) there is no single, clear, right answer to the problem.

The following are some examples of ethical quandaries that take courage and that involve the behavior of other counseling professionals: What if you know that one of your professors has published, under her or his own name only, an article based largely on a paper you wrote? What if your supervisor at your internship site is engaging in a behavior that you strongly believe is unethical? What if you know that one of your fellow interns, who is also your friend, is engaging in inappropriate relationships with clients? In such instances, it can be easier to turn away from the problem than to confront the individual involved and run the risk of retaliatory action by the professor, a poor evaluation from your supervisor, or the loss of a friend. Chapter 8 discusses important points you must consider if you suspect that another professional is behaving in an unethical manner and actions you might take.

It can be easier to recognize ethically questionable behavior on the part of others than to acknowledge that you may also have some vulnerabilities. Consider the extent to which you might be at risk of taking these potential ethical mis-steps that involve your own behavior: What if you know that you are supposed to maintain personal boundaries between you and your clients, but just once you agree to allow a client to buy you a cup of coffee and have a social conversation after a session has ended? What if you know that you are supposed to render diagnoses of mental and emotional disorders for your clients based on criteria in the *Diagnostic and Statistical Manual of Mental Disorders*, Fifth Edition–TR (American Psychiatric Association, 2022), yet you generally render the same diagnosis of *adjustment disorder* for most clients because you think this diagnosis is the least stigmatizing? What if you report to a counselor certification board that you attended a continuing education workshop you paid for, even though you did not actually attend it? In these situations, it might be tempting to make some minor compromises to your usual ethical behavior, especially when you feel that no harm will come to a client or to anyone else as a result.

Nonetheless, if you do nothing when you know the behavior of other professionals is unethical, or if you compromise your own ethical behavior, you may be setting foot on an ethical slippery slope. The *slippery slope phenomenon* is a term used by moral philosophers to describe what happens when one begins to compromise one's principles: It becomes easier and easier to slide down the slope, diminishing one's sense of moral selfhood along the way.

The diagram of the model also includes external forces that can support counselors in their efforts to maintain sound, professional practice. External sources of guidance and support include consulting with colleagues, seeking supervision, and increasing your knowledge and skills through continuing education activities. Your code of ethics is certainly a major source of guidance. Some laws support counselors in fulfilling ethical obligations; for example, privileged communication statutes can help you to uphold your clients' confidentiality when called to testify in court or produce records of counseling sessions. The system (school, agency, institution, or organization) in which you are employed may also have policies on which you can rely when confronted with a challenge or a request to compromise your ethics.

PROFESSIONAL ETHICS

Concern about ethics acknowledges the awesome responsibilities inherent in the practice of counseling. A counselor's work can make a difference in whether an abused child's life situation is recognized and addressed, whether a battered spouse finds the self-affirming courage to move to a safe environment, or whether a clinically depressed client who has suffered multiple losses of loved ones as a result of the COVID-19 pandemic finds the hope needed to move forward in life. Other clients come with less dramatic, more mundane problems, yet counseling can play a vital role in their struggle to lead more meaningful and effective lives (Pope & Vasquez, 2021). Ethical counselors take these responsibilities seriously.

Foundations of Ethics

For many centuries, philosophers have debated what characterizes a moral and ethical person and how to behave in a moral and ethical manner, and these issues have been addressed within the helping professions since ancient times. The Hippocratic Oath was written about 2,500 years ago in ancient Greece, and in fact, Greek philosophers such as Plato and Aristotle created most of the ethical principles that helping professionals use today.

Theories of Ethics

Ethical theories provide a framework that counselors can use to decide whether an action or contemplated action is ethical. It is important for you to have an ethical theory because it will enable you to resolve the ethical dilemmas you encounter in your work and help you defend the solutions you reach. However, a number of ethical theories take opposing positions on what it means to be and act as an ethical person. Having some familiarity with a few of these positions may help you become aware of the approach you take in your ethical decision making as a counselor and perhaps challenge the assumptions you make. Remember that ethical reasoning is an acquired skill, not an inherent gift, and it can be sharpened through practice.

One set of opposing viewpoints on ethics is *ethical absolutism* versus *ethical relativism*. Ethical absolutists believe that there are some absolute moral standards that are universally applicable; that is, these standards must prevail in all circumstances and are not dependent on a person's beliefs or cultural values. These standards exist *a priori* (they exist before a situation arises) and independently of whether one believes in them. Ethical relativists, on the other hand, do not believe that any absolute moral standards exist that can be universally applied. Rather, they take the position that if the members of a culture believe an action is morally right, then it is morally right to perform that action in that culture (Freeman, 2000). As you begin to study the codes of ethics for counselors, these codes may seem to you to be written in absolutist terms, such as "Counselors do not . . ." and "Counselors ensure that . . .," which appear to suggest that there are absolute *dos* and *don'ts* of ethical behavior. We believe, however, that ethical standards must be interpreted in a relativistic manner, taking into account the uniqueness of the client, the situation, and any cultural variables that are involved. These distinctions should become clearer to you as you progress through the chapters of the text and begin to grapple with the ethical issues and dilemmas that are presented.

A related issue that is raised by philosophers of ethics is that of *utilitarianism* versus *deontology*, or consequential versus nonconsequential ethics. Utilitarian thought, represented by thinkers such as John Stuart Mill, argues that people should choose the act that will do the greatest good for the greatest number of people. In other words, an act is evaluated by its consequences. By contrast, deontologists, represented by the thinking of Immanuel Kant, believe that an action is justified by its being inherently right, not by its consequences. Another way to state this idea is that what makes an action right is the principle that guides it. This philosophical issue underlies much of the reasoning that counselors use in attempting to determine what ethical professional behavior is.

A third set of opposing philosophical viewpoints has to do with what motivates people to act morally or ethically. *Egoism* is the term used to describe actions taken out of self-interest, whereas *altruism* is the word that describes actions taken to benefit others (Freeman, 2000). Most people who choose counseling as their life's work tend to see themselves as altruists, and indeed one of the most fundamental ethical values of counselors is that "client welfare comes first." Although this ethical value is well established in the counseling profession, this does not mean that there is no place for egoism or self-interest in our work. When we consider the possible consequences of a decision or action we might take, we would be prudent to reflect on the effects that such an action could have on us as well as on our clients. This dual consideration of altruism and self-interest, in fact, is reflected in the differences between the ethical and the legal perspectives that are presented throughout this text. The ethical perspective is focused more on the welfare and protection of the client, whereas the legal perspective is focused more on protecting the counselor.

1-3 The Case of Edward

Edward is a high school counselor. His administrative supervisor is the school principal, Ms. Gomez. Although Ms. Gomez has no training as a counselor, she generally has been supportive of the counselors on her staff. She asks Edward to provide, for her eyes only, a list of his clients and their presenting concerns. Edward trusts his supervisor to be responsible and to refrain from sharing the list with others. Nonetheless, Edward believes it would be wrong to produce the list because it would violate his clients' right to confidentiality. At the same time, he realizes he could be at risk for disciplinary action for refusing to produce the list. In the end, he decides that no real harm would be likely to result from giving the list to Ms. Gomez. He is also concerned that a refusal could negatively affect Ms. Gomez's supportive attitude toward the counselors.

- What should Edward do? Do you believe that the *principle* underlying confidentiality should be the overriding consideration?
- Or do you believe that it is more important for Edward to consider the *consequences* of his decision?

Discussion: If Edward reasons that the ethical principle is more important, he would be committing himself to uphold the moral principle of fidelity. *Fidelity* refers to fulfilling a responsibility of trust in the counseling relationship: Counselors strive to be faithful to the promises they make, such as keeping clients' disclosures confidential. If Edward adheres to this line of reasoning, he could be said to be thinking as an ethical absolutist—i.e., that the principle always applies, regardless of the situation. He would also be thinking as a deontologist, by deciding that keeping the students' confidentiality is the right thing to do, regardless of the consequences. In addition, he might be relying on altruism, in that he believes that his actions must uphold client welfare rather than serve his own interests.

If Edward decides to produce the list for Ms. Gomez, he might be motivated by egoism, or a focus on protecting himself and his fellow counselors from negative repercussions. He could be using utilitarian reasoning as suggested by Mill: that because no harm is likely to come to the students, his decision will do the greatest good for the greatest number of people—not only himself and his fellow counselors but also the students, who would be better served by having a supportive school administration.

We believe the best course of action in this situation would be for Edward to have an open discussion with his principal and explain his concerns about providing her with the list of students he has seen in counseling. We would hope that his principal would either understand his concern and withdraw her request or convince Edward of her need for the list and promise to keep it confidential, which would override his ethical concerns about the privacy of his students.

All the theories discussed up to this point have focused on the question of what constitutes ethical action. Other theories focus on what constitutes ethical character. Virtue ethics, which originated with Aristotle, explores the question of what character traits or dispositions form the basis for right action (Wilczenski & Cook, 2011). Aristotle believed that positive personal character is developed when individuals consistently take actions that are based on their society's values. Virtue ethics thus focuses on individuals rather than on actions and evaluates the whole individual instead of isolated decisions the individual makes. Virtue ethicists believe that moral choices cannot be made by issuing rules; what is needed instead is good judgment.

The ethics of care, or relational ethics, is based on the recognition that human beings exist in relationship and connection with one another. Psychologist Carol Gilligan (1982), who represents this perspective, believes that ethics exists in a world of relationships in which the awareness of connections among people gives rise to a recognition that we are responsible for each other. Thus, the solution to an ethical dilemma is not found in a contest between opposing philosophies but, rather, in a strengthening of the relationship on which the actors in the dilemma depend. Feminist ethicists have further articulated the ethics of care. Manning (1992) has stated, "An ethic of care involves a morality grounded in relationship and response.... In responding, we do not appeal to abstract principles... rather we pay attention to the concrete other in his or her real situation [and to] the effect of our response on the networks that sustain us both" (p. xiv). Relational or feminist ethicists do not disagree with principle ethicists, but the focus of the former is different: They view moral actions as those that empower individuals, promote social justice, and ensure that all people are cared for and nurtured to develop their potentials (Jordan & Duffey, 2020).

With these general ethical theories in mind, we now turn to a consideration of ethical reasoning as it has been applied in the field of counseling.

Linking Theory to Practice: Principles and Virtues

Thoughtful mental health professionals have struggled for many decades with questions of ethical ideals, concepts, principles, and values and on how to link these to ethical decisions in professional practice (Beauchamp & Childress, 1994; Jordan & Meara, 1990; Kitchener, 1992; Levitt et al., 2015; Meara et al., 1996). Two helpful perspectives are *principle ethics* and *virtue ethics*. Even though these two approaches are quite different from one another, they are complementary. When integrated into a holistic framework for ethical decision making, they can serve as a bridge from philosophy to practice.

Principle ethics has its foundation in moral principles, which are agreed-upon assumptions or beliefs about ideals that are shared by members of the helping professions. They are *prima facie* obligations that are always considered in ethical decision making (Meara et al., 1996). Although moral philosophers do not agree about the nature or number of moral principles, the following six are included in the Preamble to the ACA *Code of Ethics* (ACA, 2014):[†]

- *Respect for autonomy* means to foster self-determination. According to this principle, counselors respect the rights of clients to choose their own directions, act in accordance with their beliefs, and control their own lives. Counselors work to decrease client dependency and foster independent decision making.
- *Nonmaleficence* means to avoid taking actions that cause harm. This principle, long established in the medical profession, obligates counselors to avoid taking actions that risk hurting clients, even inadvertently.
- *Beneficence* is the counterpoint to nonmaleficence. It could be argued that the obligation of ordinary citizens in our society ends with doing no harm to others, whereas professionals have the higher obligation of providing a service that benefits society. Thus, counselors actively work for the good of individuals and society by promoting the mental health and well-being of their clients.
- *Justice* refers to the counselor's commitment to fairness in professional relationships by treating people equitably. Counselors' actions and decisions must be fair to all concerned. Justice demands equality, which has implications for nondiscrimination and equitable treatment of all clients.

[†] *Source*: Based on Preamble to the ACA *Code of Ethics* (2014), American Counseling Association.

- *Fidelity* refers to fulfilling a responsibility of trust in the counseling relationship by honoring commitments and keeping promises. Counselors strive to be faithful to the promises they make, such as keeping clients' disclosures confidential.
- *Veracity* means truthfulness and addresses the counselor's obligation to deal honestly with clients and others with whom they relate professionally.

In theory, all these principles have equal value and should be considered along with all the others when weighing an ethical decision. In reality, however, these principles can compete with one another, and counselors may need to sacrifice one in order to uphold another. For example, a counselor of a client who expresses suicidal ideation may decide to intervene by notifying family members against the client's wishes (thus breaching confidentiality and sacrificing fidelity) or by seeking involuntary hospitalization (thus sacrificing client autonomy) to uphold the obligations to prevent harm and do good (nonmaleficence and beneficence).

Virtue ethics starts from a premise very different from that of principle ethics. The basic assumption of virtue ethics is that professional ethics involves more than moral actions: It also involves traits of character or virtue. Virtue ethics focuses on the actor rather than on the action. Principle ethics asks, "What should I do?"; virtue ethics asks, "Who should I be?" Patterns of virtuous behavior are evident throughout the career of a professional, rather than being found only in a particular action or decision. Thus, this perspective asks you to look at who you are, rather than at what you do. Certain characteristics of virtuous agents have been suggested as appropriate for mental health professionals (e.g., Meara et al., 1996). We hope that you will consider whether these characteristics represent the ideals you hold for yourself and that you will assess their relevance for you as an aspiring counselor.

- ***Integrity.*** Virtuous agents are motivated to do what is right because they believe it is right, not because they feel obligated or fear the consequences. They have stable moral values and are faithful to these values in their actions.
- ***Discernment.*** Discerning counselors can perceive the ethically relevant aspects of a situation, know what principles apply, and take decisive action. Discernment involves a tolerance for ambiguity, the ability to maintain perspective, and an understanding of the links between current behaviors and future consequences.
- ***Acceptance of emotion.*** Without discounting the values of logic and systematic deliberation about ethical issues, virtuous agents also recognize the role of emotion in ethical decisions. Rather than assume that emotion hinders reason, they believe that emotion informs reason. Virtuous counselors are compassionate and sensitive to the suffering of others.
- ***Self-awareness.*** Virtuous agents know their own assumptions, beliefs, and biases and how these may affect their relationships and interactions with others.
- ***Interdependence with the community.*** Virtuous agents realize that values cannot be espoused without awareness of context. They are connected with and understand the expectations and values of their communities.

1-4 The Case of Gabriela

Gabriela has been counseling a 15-year-old girl, Danielle, for several weeks. Gabriela has had to work hard to gain Danielle's trust and attributes some of her success to herself being a parent of a teenage daughter. Danielle was raised by abusive parents until she was 13, when she went to live with her grandparents. Today she tells Gabriela that she is having some problems with her boyfriend.

As Danielle describes these problems, Gabriela realizes that the boyfriend is treating Danielle in an abusive manner. When Gabriela expresses her concern about this, Danielle replies that she loves him and can get him to change, that her grandparents don't know about his behavior, and that she absolutely does not want Gabriela to tell them or anyone else.

- How might you apply each of the six moral principles to this situation? In this case, which moral principle do you think must take precedence? How might you apply virtue ethics to this dilemma?

Discussion: The moral principles could be applied in several ways. The principle of respect for autonomy would support deferring to Danielle's wishes and not telling anyone about her abusive boyfriend. The principle of nonmaleficence would require Gabriela to weigh the risk for harm. Telling someone who could prevent further abuse would support nonmaleficence. However, if Danielle becomes upset by this action and refuses to continue in counseling, telling someone about her abusive boyfriend also could violate the principle of doing no harm. Likewise, beneficence could be interpreted in more than one way: Beneficence means that professionals have an obligation to provide a service that benefits society, so taking an action that would stop the abuse would best serve society's interests. Yet beneficence also means promoting clients' mental health, and Gabriela could respect Danielle's wishes and continue to work to increase her awareness and self-esteem, which, for now, could be the most beneficent thing to do. How Gabriela adheres to the principles of justice, fidelity, and veracity will depend largely on whether she has informed Danielle that she would have to breach confidentiality to prevent harm to her. If so, then telling her grandparents would be just and truthful and would not be breaking her promise to keep Danielle's disclosures confidential.

If Gabriela reasons through her dilemma from the perspective of virtue ethics, she will put primary importance on preserving and nurturing her relationship with Danielle. She will also be concerned about Danielle's other supportive networks, such as her relationship with her grandparents and what she considers to be a loving relationship with her boyfriend, despite his problem behaviors. Gabriela will work collaboratively with Danielle in reaching the decision as to whether or not to involve the grandparents. Gabriela may be concerned about the risks to herself if she honors Danielle's wishes, if Danielle is harmed by her boyfriend, and if the grandparents threaten Gabriela with a complaint to her licensing board or a lawsuit. Nonetheless, she will act with integrity, doing what she believes is right despite the potential consequences. She will be sure to tune into her own feelings, checking to see whether her values and life experiences might be influencing her desire to protect Danielle.

Both perspectives—a focus on principles and a focus on virtues—can contribute to your understanding of the basis for professional ethics. Principle ethics help you to systematically evaluate what you should do when trying to resolve an ethical dilemma. Virtue ethics can help you examine your ideals and define the kind of person you aspire to be as a helping professional. Thinking about principles and virtues requires you to look inward in order to identify internal resources that can assist you in ethical decision making. There are external resources as well, and primary among these is your professional code of ethics.

Codes of Ethics

Promulgating a code of ethics is one way that a group of practitioners can establish its professional status. Codes of ethics serve several other important purposes as well. They educate members of the profession as well as prospective clients about what constitutes ethical practice, help to ensure

accountability through enforcement of the standards, protect the profession from government intrusion by allowing the profession to regulate itself and function autonomously, promote stability within the profession by establishing agreed-upon guidelines for practice, and serve as a catalyst for ongoing improvement of practice (Cottone et al., 2022; Herlihy & Corey, 2015a). An established code of ethics also can protect practitioners—if professionals behave according to established guidelines, their behavior is more likely to be judged in compliance with accepted standards in a malpractice suit or licensing board complaint. Most fundamentally, codes of ethics exist to protect and promote the welfare of clients.

Some counselors practice *mandatory ethics*; that is, they function at a level of ethical reasoning that merely keeps them in compliance with minimal standards. By complying with these basic *musts* and *must nots*, they meet the letter but not the spirit of the ethical standards. Those who practice at this level of ethical functioning may be motivated by a desire to prevent lawsuits, complaints to ethics committees or licensing boards, or getting into trouble in some other way. According to Pope and Vasquez (2021), counselors who set their sights at this level are vulnerable to denial and to other means of distorting, discounting, or dismissing the ethical questions they encounter. Some of the self-statements that these counselors use to justify their actions include:

"It can't be unethical if I don't see it as an ethical issue."

"It isn't unethical if there is no ethical standard that specifically prohibits it."

"It can't be unethical if I know other practitioners who do it."

"It isn't an ethical problem as long as no client has ever complained about it."

"It's not unethical as long as no one finds out about it."

Other counselors practice *aspirational ethics*, a term that describes the highest standards of conduct to which counselors can aspire. They understand the spirit behind the code and the moral principles on which it rests. They not only look outward to established standards but also look inward and ask themselves whether what they are doing is best for their clients. Aspirational ethics, or *positive ethics* (Corey et al., 2024), means striving for the highest level of care for clients and is closely related to the concept of best practice. Rather than focusing on preventing wrongdoing and disciplinary actions, the emphasis is on continually striving to do better to serve clients.

A code of ethics that would address every possible situation that a counselor might encounter would probably fill an entire library. Thus, you cannot expect your code of ethics to provide an answer to every question you might have (Birrell & Bruns, 2016). Codes are a crucial resource, but they are not a substitute for an active, deliberative, and creative approach to fulfilling your ethical responsibilities (Pope & Vasquez, 2021). You must attend to both the letter and the spirit of the code and work to understand the intentions that underlie each standard. As Herlihy and Corey (2015b) have noted, "There is a very real difference between merely following the ethics code and living out a commitment to practice with the highest ideals" (p. 13).

Your primary professional association, the ACA, has established a code of ethics to guide you in your practice. The ACA *Code* can be found in Appendix A and at www.counseling.org. Figure 1-2 presents a brief overview of the nine sections of the *Code* and their general provisions.

The current ACA *Code of Ethics*, adopted in 2014, is the seventh version of the ethics code established by ACA and its predecessor organizations. Development of the first *Code* was initiated in 1953 by Donald Super, then president of the newly formed American Personnel and Guidance Association (APGA). The *Code* was adopted in 1961. It was revised in 1974 and has been revised approximately every 7 to 10 years since that time. The current *Code* is the result of a lengthy revision process that began in 2011, when a *Code* revision task force was appointed to revise the 2005 *Code* that was then in effect (Francis, 2015). In 2023, an Ethics Review Work Group was formed to develop

FIGURE 1-2 ACA *code of ethics*: a synopsis

Preamble describes the American Counseling Association and its members, defines counseling, and enumerates core professional values and ethical principles.

Purpose sets forth six purposes of the *Code of Ethics* and describes the ethical decision-making process.

Section A: The Counseling Relationship addresses important issues in forming, maintaining, and ending the counseling relationship. This section includes guidelines to help counselors keep client welfare foremost. It also contains standards that emphasize the importance of informed consent and of preventing the harm that can be caused by imposing one's own personal values onto the counseling relationship. It provides guidance on how to maintain appropriate professional boundaries, advocate for clients, manage fees and business practices, and terminate the counseling relationship. Standards are also provided for working with multiple clients, groups, and clients served by other professionals.

Section B: Confidentiality and Privacy addresses the client's right to privacy regarding the information shared during counseling sessions and records of sessions. Exceptions and limitations to confidentiality are specified, and special considerations in working with families, groups, and clients who lack capacity to give informed consent are addressed. Guidelines are also offered for maintaining confidentiality when consulting.

Section C: Professional Responsibility contains standards related to competence. It emphasizes the importance of advertising services and qualifications in an accurate manner. It also addresses the counselor's responsibilities to the public and other professionals and offers cautions regarding treatment modalities.

Section D: Relationships with Other Professionals offers guidelines for relationships with colleagues, employers, employees, and consultees. This section highlights the importance of respecting and establishing good working relationships with professionals in related mental health professions.

Section E: Evaluation, Assessment, and Interpretation includes standards on competence to select, use, and interpret assessment instruments. Client rights in testing, test security, and proper testing conditions are addressed. This section also includes standards related to diagnosis of mental disorders and forensic evaluations.

Section F: Supervision, Training, and Teaching presents guidelines for counselor supervisors, trainers, and educators. Responsibilities of supervisors and counselor educators are elucidated, and standards address relationship boundaries, evaluation and remediation, endorsement of students to enter the profession, and student welfare.

Section G: Research and Publication describes research responsibilities, rights of research participants, and the reporting of research results. A range of issues is covered, from protection of human subjects to ethical procedures in seeking publication.

Section H: Distance Counseling, Technology, and Social Media presents guidelines to assist counselors to best serve clients using distance counseling, technology, and social media. This section addresses ethical issues that are specific to these new and emerging resources.

Section I: Resolving Ethical Issues addresses the responsibility of counselors to know ethical and legal standards and explains procedures for resolving and reporting suspected ethical violations.

Glossary of Terms provides definitions of terms used in the *Code*.

recommended revisions to the 2014 *Code*. Typically, the process of revising the *Code*, from forming the work group to ratification by the ACA Governing Council, takes approximately 3–4 years.

As you learn about the ethical standards that you will be expected to uphold, keep in mind that codes of ethics are living documents that change over time. They are periodically revised as the profession's knowledge base grows and as consensus emerges about new and controversial ethical issues. Although the fundamental ethical principles do not change, new questions are constantly arising as to how to apply the principles in a changing world of counseling practice. For instance, when computer technologies first became widely available, ethical concerns centered around the

security of client information stored on computers. Later, as internet usage burgeoned, many questions arose around the ethics of internet counseling. The explosive increase in the use of social media raised a different set of issues, such as whether counselors should *friend* clients, former clients, or supervisees. These developments led to the creation of a separate section on distance counseling, technology, and social media (ACA, 2014, §H) that reflects the myriad issues that arise when counselors use electronic means to provide services, store records, advertise their services, and communicate with clients (Francis, 2015). This section of the *Code* gained importance during the COVID-19 pandemic, when virtual counseling became the norm.

It is also important to keep in mind that, historically, counseling has not been a unified profession. Codes of ethics have proliferated as various specialty groups within counseling (Brown et al., 2017; Kelly, 2001), certification bodies, and state licensure boards have developed their own ethical standards. When you are established in your own professional practice, it is likely that you will hold multiple affiliations and will be bound to know and adhere to multiple codes. For instance, you might be a member of ACA and several of its divisions that have published specialty guidelines or separate codes of ethics (for example, the Association for Specialists in Group Work published the *Guiding Principles for Group Work* in 2021), be a national certified counselor (NCC), and be licensed as a counselor in your state. Holding each of these credentials will require you to abide by each entity's particular code of ethics. Although these codes do not contradict each other, the existence of multiple codes of ethics has created difficulties in enforcement, confusion for users of counseling services, and confusion for counseling professionals themselves (Herlihy & Remley, 2001). Efforts are being made by various organizations to bring standards into alignment; for example, the ACA Ethics Committee reviews the codes of ethics and guidelines for best practice of its divisions to ensure that they are consistent with the ACA *Code of Ethics*. Yet until a single, universally accepted code of ethics for the counseling profession is established, you will need to be knowledgeable about all the codes of ethics that pertain to your practice. Initially, though, what is important is that you become familiar with your primary code, which is the ACA *Code of Ethics* (2014).

EXPLORING THE ACA *CODE OF ETHICS* We suggest that you turn to Appendix A and peruse the ACA *Code*. You will see that it is a lengthy and detailed document, which could give you the impression that it must contain the answers to almost any ethical question you may have. To test that impression, use the *Code* to try to find the answers to these questions:

- Arriving home after a difficult counseling session at the end of the day, you feel a need to vent with your life partner about your frustrations over the client's lack of progress. Would it be ethical for you to unburden yourself in this way?
- One of your clients has been laid off from work and requests that you barter with her (exchange services) until she can find a new job. Would it be ethical for you to agree to barter with this client?
- A client presents you with a gift of a power pod to use with your laptop computer. Would it be ethical for you to accept this gift?
- A client tells you that they are moving to another state and wants you to continue to work with them via distance counseling. Would it be ethical for you to agree to continue to counsel this client using the internet?

As you completed this exercise, you probably found that the answer to each question is, like the answers to the vast majority of questions about ethical issues, "It depends." In researching the first scenario, you may have explored Section B of the *Code*, which addresses confidentiality, to see whether it would be permissible for you to vent your frustrations with your life partner. You learned the importance of not making any disclosures that could identify the client, but to monitor your effectiveness (§ C.2.d.), you would still need to consider the larger question of acceptable ways to deal with unwelcome feelings toward a client.

Although § A.10.e. directly addresses bartering and § A.10.f. addresses gifts, neither standard provides a "yes or no" answer; rather, these standards suggest factors to consider when deciding whether to agree to enter into a bartering arrangement or whether to accept a gift from a client. The final scenario is more complicated; you may have found some guidance in Section H, which addresses distance counseling, but legal issues exist as well and will need to be explored.

These scenarios illustrate the necessity of developing ethical reasoning skills, as opposed to relying on a code of ethics to provide answers to real-world dilemmas. Ethical decision making is a complex process, as is further discussed in the next section.

ETHICAL DECISION MAKING Ethical decisions are rarely easy to arrive at, and dilemmas can be very complex. When counselors encounter ethical dilemmas, "they are expected to engage in a carefully considered ethical decision-making process" and use a "credible model of decision making that can bear public scrutiny of its application" (ACA, 2014, Purpose). Although no particular decision-making model has been shown to be universally effective or applicable, many models do exist, and we briefly review them here for your consideration.

One of the earliest models was *A Practitioner's Guide to Ethical Decision Making* (Forester-Miller & Davis, 1995). This guide presented a practical, sequential, seven-step model that was based on the moral principles already discussed in this chapter. Over two decades later, Koocher and Keith-Spiegel (2016) presented a nine-step model that took a similarly logical and primarily cognitive approach to ethical decision making. The assumption inherent in these models—that ethical decision making is an objective process for which the professional is solely responsible—has been challenged by some writers, particularly those with a feminist orientation (Hill et al., 1995; Meara et al., 1996; Rave & Larsen, 1995). Feminists have cautioned that traditional ethical decision-making models represent the information-processing style of White males, in that the models are linear, logical, rational, dispassionate, abstract, and paternalistic. Feminists have suggested that ethical decision making that is also holistic, intuitive, emotional, compassionate, personal and contextual, and mutual may be more inclusive of other processing styles and more culturally appropriate. Feminist theorists have also emphasized the importance of remembering that ethical decision making does not occur solely within the mind of the professional. Walden (2015) urged including the client in the process, noting that clients are empowered when counselors make ethical decisions *with* them rather than *for* them. This perspective is sometimes called participatory or relational ethics. Counselors who practice participatory ethics invite clients to be co-contributors to the ethical decision-making process. They reject the premise that the counselor is the keeper of knowledge and the person responsible for any decision related to an ethical issue that arises in the counseling process (Fine et al., 2021).

Early models were also criticized for neglecting to consider multicultural issues, which led to the development of several models that emphasized cultural sensitivity. Garcia et al. (2003) offered a transcultural, integrative model as being more appropriate when working with clients from diverse cultural backgrounds. They incorporated virtue ethics along with the feminist concepts of reflecting on one's own feelings and balancing the perspectives of all involved. They suggested that a vital component that should be included early in the decision-making process is for counselors to reflect on their own worldviews and on how these affect their interpretation of an ethical dilemma. Frame and Williams (2005) presented a culturally sensitive ethical decision-making model that, like the feminist models, was based in an ethic of care and a consideration of power dynamics. To increase multicultural sensitivity, they added the element of assessing acculturation and racial identity development of the counselor and the client. Herlihy and Watson (2006) offered a model based on a social justice perspective that puts multicultural competence at the core of the ethical reasoning process. The model is grounded in virtue ethics, cultural identity development, and collaborative

decision making. Tarvydas et al. (2015) proposed an applied participatory ethics model that is based in social justice principles.

Cottone (2001) proposed a social constructivist model. Social constructivism is a relatively recent movement in the mental health field and purports that a person cannot know reality through individual contemplation because reality does not exist as objective fact. Rather, reality is socially constructed through interactions with others. Social constructivists see ethical decision making not as a process that occurs in the mind of the decision maker but as a process that is always made in interaction with at least one other person and that involves negotiating and consensualizing (Cottone, 2001). Specialized models that are focused on counseling specific populations or on particular ethical issues have also been promulgated (Deroche et al., 2015). Models have been offered for managing boundary issues (Gottlieb, 1993; Herlihy & Corey, 2015b), resolving value conflicts (Kocet & Herlihy, 2014), integrating spirituality and religion into counseling (Barnett & Johnson, 2015), treating eating disorders (Matusek & O'Dougherty, 2010), school counseling (Brown et al., 2017; Luke et al., 2017), counseling military personnel (Prosek & Holm, 2014), practicing play therapy (Seymour & Rubin, 2006), and (for counselor educators) addressing problematic student behaviors (Letourneau, 2016), among others.

Clearly, many ethical decision-making models exist, and we believe that no one model will be the most appropriate for every counselor or for every situation. Johnson et al. (2022) examined 38 existing models and organized them into a taxonomy, with the intent of providing counselors with a streamlined way to choose the best model in a given situation. It is important that you are familiar with several models so that you can select the model or models that are most helpful for the ethical dilemma you are facing. There is some evidence that, in practice, counselors may not be making full use of ethical decision-making models (Brown & Armstrong, 2022; Burkholder et al., 2020; Johnson & Purgason, 2021; Levitt et al., 2015). This practice could lead to problems if a counselor made what turned out to be a poor decision and a client was harmed. Responding to a complaint filed with the ACA or with a licensing board will be difficult if the counselor cannot demonstrate the use of a "credible ethical decision-making model" as required by the ACA *Code of Ethics* (2014). When you are faced with a difficult dilemma, you can do much to protect yourself by implementing the actions suggested in a model, being able to provide a rationale if you deviate from the model, and documenting your decision-making process.

COMMON ELEMENTS IN DECISION-MAKING MODELS There is much to be learned from all the existing ethical decision-making models, and we do not endorse any one particular model as being the *right* one for everyone or in every circumstance. Instead, what follows is a description of steps that many of the models seem to have in common. We have tried to incorporate lessons that can be learned from principle and virtue ethics, feminist and multicultural ethics, social constructivism, and specialty models. We caution you to keep in mind that although a list of steps suggests that ethical decision making is a linear progression, in reality counselors rarely follow a set sequence of steps to resolve an ethical dilemma. In practice, numerous aspects of ethical decision making occur simultaneously in a dynamic process (Woody, 2013). What we offer next is not a step-by-step model; rather, we have identified common elements of the decision-making process that are found in multiple models. Although we believe that any credible model should contain these elements in some form, this may not always be the case. To help ensure that you have been thorough, you might use this checklist after you have applied your selected model to a dilemma you are resolving.

Identify and define the problem. Before deciding what action to take when faced with a dilemma, determine whether you have an ethical or a legal or clinical issue. If a legal issue is involved, consult with an attorney. If you have a clinical issue, consult with your supervisor or

a trusted colleague. If, indeed, you have an ethical dilemma, it is prudent to take time to reflect and gather information. Although you may feel some sense of urgency, rarely will decisions that have ethical dimensions have to be made immediately. Take the time to consider what you know (or what you can find out) about the situation, applicable ethical guidelines, and any laws that might be relevant. Try to examine the problem from several perspectives, and avoid searching for simplistic solutions.

Involve your client in the decision-making process. This is not a separate step in ethical decision making; rather, it should occur throughout the process. Walden (2015) reminded counselors that the client is an integral part of the ethical community of the counseling relationship. Including clients in the process both empowers them and is culturally appropriate practice. We can think of very few situations that would preclude making the client an active partner in decisions affecting that client.

Review relevant codes of ethics and the professional literature. Examine the codes of ethics of the professional organizations to which you belong (as well as the ethical standards of your state licensing board if you are licensed as a counselor) to see if your issue is addressed in them. Be sure to read the codes carefully, as there may be several standards that pertain to different aspects of the dilemma. Also, read the recent literature on the issue at hand. This will help to ensure that you are using the most up-to-date professional knowledge on the issue (Herlihy & Corey, 2015a).

Consider the principles and virtues. Reflect on how the moral principles apply to the problem. Identify any ways they may compete with each other, and rank them in order of their priority in this situation. Consider how virtue ethics might apply in the situation as well. Rather than focus exclusively on what you need to *do* in the situation, also reflect on who you want to *be* and how any possible action might affect your sense of moral selfhood.

Tune into your feelings. Virtue ethicists believe that emotion informs judgment. Your feelings will influence how you interpret the dilemma, so it is important to consider what emotions you are experiencing as you contemplate the situation and your possible actions. To what extent are you being influenced, for instance, by emotions such as fear, self-doubt, or an overwhelming sense of responsibility? Being aware of your emotions, beliefs, values, and motivations can help guide you in your decision making.

Consult with colleagues or experts. Decisions made in isolation are rarely as sound as decisions made in consultation. Corey et al. (2024) have pointed out that poor ethical decision making often stems from our inability to view a situation objectively because we are emotionally invested in it or because our prejudices, values, or emotional needs are clouding our judgment. In addition, consultation would serve as an important element of your defense in court if your decision were challenged legally (Wheeler & Bertram, 2019).

Consider the context. Keep in mind that your worldview will affect how you interpret the dilemma and that the client's worldview and culture may differ from your own. The resolution that is chosen for the dilemma must not only feel right to you but also be appropriate for the client. It is also important to remember that decisions occur in a context. Therefore, it is useful to reflect on the potential ramifications of a decision for the client's family members, the community, and other professionals.

Identify desired outcomes, and consider possible actions to achieve the outcomes. Even after thoughtful consideration, a single desired outcome rarely emerges in an ethical dilemma. There may be several outcomes you would hope to see achieved in a situation. Consider possible actions that you could take to achieve the desired outcomes. It may even be useful to list

desired outcomes on one side of a page and on the other side to generate possible actions that would facilitate the achievement of each of those outcomes. It is possible that implementing a particular action may achieve one desired outcome while eliminating another desired action, thus forcing you to prioritize and choose one outcome at the expense of the other. Ponder the implications and consequences of each option for the client, for others who will be affected, and for yourself.

Choose and act on your choice. Once you have selected an action or series of actions, check to see whether your selected options are congruent with your ranking of the moral principles. Pay attention to how you feel about your choice. This final step involves strengthening your ego or gathering the moral courage to allow you to carry out your decision.

SELF-MONITORING Even after the most careful deliberation, conscientious counselors cannot help but ask, "How can I know whether I've done the right thing?" You can apply several self-tests after you have resolved an ethical dilemma. The first three tests, suggested by Stadler (1986), have withstood the test of time. First is the test of *justice*, in which you ask whether you would treat others the same way in this situation. Second is the test of *universality*, which considers if you would be willing to recommend the course of action you followed to other counselors who find themselves in a similar situation. Third is the test of *publicity*: Are you willing to have your actions come to light and be known by others? Another test is the *reversibility* test, which is a version of the Golden Rule; in this test, you ask yourself if you would have made the same choice if you were in the client's shoes or if your child or life partner were subject to that choice. Finally, you can check for *moral traces*, which are lingering feelings of doubt, discomfort, or uncertainty that counselors may experience after they have resolved an ethical dilemma, particularly when expediency, politics, or self-interest has influenced the decision. Moral traces are unpleasant but perform an important function: They act as a warning sign that you may have set foot on an ethical slippery slope, as defined earlier in this chapter.

We hope you will return to this material on the ethical decision-making process as you ponder the case studies that are presented throughout this text. As you reflect on what you might do if you were the counselor in each case study, you can gain practice in applying a systematic model, as required by your code of ethics. Being an ethical professional involves a combination of knowledge, ethical reasoning skills and problem-solving strategies, a basic understanding of philosophical principles, application of a credible model to dilemmas that arise, and self-monitoring to ensure that you respond with maturity, judgment, and wisdom. It is a task that requires a life-long commitment and is never really finished. Even the most experienced counselors who are intimately aware of the ethical standards wrestle with difficult ethical issues and dilemmas.

LEGAL ISSUES

The discussion of professional ethics that precedes this section emphasized the serious responsibility you have to clients, the difference you can make in their lives, and the duty you have to practice in an ethical manner. Understanding the legal system and your role in it, the legal rights of your clients, and your legal responsibilities to clients is also essential to practicing counseling in a professional manner. Legal issues within counseling are somewhat frightening because the law is an area that is complex, often vague, threatening, antithetical to the nature of counselors, and difficult to fully grasp. The process of decision making around legal issues is presented in this chapter. In addition, throughout this text, you are given information about the legal dimensions that surround your functions as a professional counselor.

Origins of Law

There are several sources of law in the United States. The basic source is the U.S. Constitution. The 50 states, the District of Columbia, and the U.S. possessions also have constitutions. Laws created by the federal, state, district, and possession governments cannot violate either federal or state constitutional principles. The United States has adopted English common law, which includes a set of societal principles that were not written into documents but have been accepted over time as obvious within U.S. society. An example of common law that is very important to counselors is the law of torts. Tort law relates to the principle that individuals will be held responsible for any harm they cause to other members of society. Malpractice is an area of tort law that holds professionals accountable for any harm they might cause to the public. The public relies on professionals to provide services in a manner that benefits and does not harm them.

A primary source of law is statutes passed by federal and state legislatures. These statutes may modify the common law but may not violate constitutional principles. Governmental regulations, both federal and state, are procedures adopted by agencies to carry out laws created by statutes. Regulations, which are created by governmental agencies, may implement statutes but may not exceed the authority of the statute. Finally, federal and state courts interpret the law. Whereas some accuse judges of creating law, courts are limited to interpreting constitutions, common law principles, statutes, and regulations.

Almost all areas of counselor practice are affected by law. Most seasoned counselors are keenly aware of the law of malpractice and the fact that they might be sued by a client. As a counselor, you will probably come to share this awareness. You also must be aware of laws related to confidentiality, records, parental rights, and licensing statutes, among others. In addition, you must be able to identify legal problems as they arise, and you must adhere to legal requirements when you are involved in any way with a legal proceeding. Legal issues are an important part of the day-to-day professional practice of counselors in all settings.

Recognizing Legal Issues

Many of the ethical and professional judgment questions you will encounter as a counseling practitioner will have legal implications as well. Sometimes counselors find it difficult to determine when they have a legal problem or what to do once a legal problem has been identified. This section of the chapter discusses how to recognize legal issues, how to get legal advice, and what steps to take to ensure proper and professional practice.

The following are examples of legal issues that counselors face in their practices:

- The secretary tells you that there is a deputy sheriff in the reception area asking for you. When you introduce yourself, the deputy hands you a subpoena that orders you to produce your case notes and any other documents related to one of your current clients.
- One of your clients asks you to come to a child custody hearing that will determine whether she will get permanent custody of her children.
- One of your clients has been arrested for drug possession. You receive a subpoena in the mail that orders you to appear at her criminal trial.
- A new client tells you that his lawyer sent him to see you. He is suing his former employer for having fired him and wants you to verify that he has emotional stress that is job related.
- A client tells you that her former husband's lawyer called and told her that she had to let her former husband have their children for the summer. She wants to know if the lawyer is right.
- A former client has sent you a letter demanding her money back. She thinks the 10 sessions she had with you were a waste of money because you did not help her. She has sent a copy of the letter to her lawyer.

- A client you are seeing is expressing suicidal ideations and refuses to go voluntarily to the local hospital for a psychiatric evaluation.
- You receive a notice from your state licensure board that a formal complaint has been filed against you and that a hearing will be held on the matter.
- In your office mail, you receive a formal legal complaint against you that has been filed with the local court, accusing you of professional malpractice. One of your clients murdered his girlfriend 9 months ago. The complaint alleges that you were responsible for the young woman's death and asks for $1 million in damages.

A simple test to determine whether there is a legal issue involved in a situation you are facing is to review the situation to see if any of the following apply: (a) legal proceedings of some type have been initiated, (b) lawyers are on the scene in some capacity, or (c) you are vulnerable to having a complaint filed against you for misconduct. If you are providing professional counseling services and one or more of these three components exist, then you definitely are dealing with a legal issue. Sometimes, all you need to do with a legal situation is clarify the nature of a counselor's role with your client and refer the client to attorneys for legal advice. When you are dealing with a legal issue and you are unsure which course of action you should take, often you will need to consult a lawyer.

Obtaining Legal Advice

Most counselors are employed by organizations or entities that provide counseling services, such as community mental health agencies, schools, businesses, hospitals, outpatient treatment programs, or colleges. These entities all have administrators and organizational structures that require the regular services of attorneys. It is the employees' responsibility to request legal advice when dealing with an issue that has legal implications beyond their ability to resolve. It is the obligation of employers to provide employees with the legal advice they need to perform their jobs appropriately.

Counselors seldom have direct access to lawyers, primarily because the cost is prohibitive. Also, administrators seek the advice of lawyers, but they must maintain their authority when making administrative decisions. When a counselor identifies a legal issue in the work setting and determines the legal questions to ask, the counselor should pose the questions to the immediate supervisor and ask for assistance. If the counselor thinks an attorney needs to have special information regarding the situation to render sound advice, the counselor should request a personal consultation with the attorney, although such consultations are not normally allowed. The supervisor will then either answer the counselor's questions based on previous experience with similar issues or seek legal advice through proper administrative channels within the organization.

In some circumstances, it is possible for counselors to give their legal problems to administrators within their agency. Many legal issues that arise are administrative in nature and should be handled by administrators rather than by counselors. Examples of legal issues that counselors might easily turn over to administrators include the following:

- A noncustodial parent demands of a school counselor that he be allowed to see his child's academic records.
- The police arrive at a counselor's door and want to see the counseling file for a current client.
- A health insurance company representative calls a counselor and wants to know why the counselor's agency has so many claims signed by the same psychiatrist.
- A client becomes irate because a counselor terminates the counseling relationship after five sessions, which is the agency's policy.
- A lawyer for a former client calls a counselor and threatens to sue if the counselor does not immediately give the lawyer the information being sought.

- A secretary who reports to an administrator appears to be revealing confidential information about clients to friends.
- A client tells a counselor that the counselor's colleague in the agency has made sexual advances during a counseling session.

If legal problems cannot be handed over to an administrator, then counselors themselves must take responsibility for resolving situations in an appropriate manner. Once counselors have disclosed their legal questions to their immediate supervisors and have received a response either from the supervisor or from an attorney advising them as to the proper course of action, counselors must follow that advice. It is essential for counselors to follow all legal advice given to them, even if they do not agree with it. Only then will counselors be indemnified by their employers and supported if problems arise later. By seeking and following legal advice when legal questions arise, counselors are taking steps that may protect them from being held individually responsible in the event their actions are challenged.

Counselors in independent private practice do not have the luxury of seeking legal advice without charge within their work environment, and they are not protected from responsibility because they do not have employers. Private practitioners must establish relationships with attorneys for legal advice just as they must retain accountants to handle their financial and business affairs. The cost of legal advice for a counselor in private practice is a necessary expense related to establishing and maintaining a business. However, finding a lawyer who understands the nature of mental health practices and is prepared to represent counselors effectively is not always an easy task (Remley, 1991). Counselors who are planning to open a private practice are advised to identify an attorney while they are establishing their business so that they will have a working relationship in place when problems arise. The best attorney probably would be a local one who is already representing one or more successful mental health practitioners—counselors, social workers, or psychologists. Such an attorney would already have been educated regarding the special issues surrounding mental health practices. If an attorney who is experienced in representing mental health professionals is not available, then a lawyer who represents other types of professionals—such as accountants, other lawyers, or physicians—would be a good alternative.

Some counselors have jobs in which they testify in court on a routine basis or provide services to clients who frequently are involved in litigation. These counselors learn their roles over time and do not need to consult attorneys for advice each time they encounter a legal situation. For most counselors, however, it is infrequent that they deal with legal proceedings, have clients who are represented by lawyers, or think they are in danger of being sued. When such situations arise for the majority of counselors, it is essential for them to obtain legal advice.

Exercising Professional Judgment

Throughout the workday, counselors often must exercise their professional judgment in difficult areas. They are held accountable for making sound and reasonable professional decisions, given the information they have available when they make such judgments (Hall & Rushing, 2010). If a client believes that he or she was harmed because of a professional decision made by a counselor, then the client might sue the counselor for malpractice. As a result, when a counselor exercises professional judgment, there is always a risk afterward of being accused of wrongdoing.

A few of the areas in which counselors make judgments that are particularly vulnerable to later legal challenges include the following:

- Determining whether a client's condition poses a danger to self or others
- Deciding what to do to prevent harm after determining that a client is a danger to self or others

- Rendering a clinical diagnosis that could have negative implications for a client at a later time
- Terminating a counseling relationship over the client's objections
- Deciding whether to enter into a counseling relationship with a client who has a problem that you have not treated before or have not been specifically trained to treat
- Reacting appropriately to a client who has expressed an interest in having a sexual relationship with you
- Using an innovative intervention that has not been empirically tested

Just as counselors must obtain legal advice when legal questions arise in their practice, they must also obtain professional consultation to the extent possible when making difficult professional judgments. It is inappropriate to ask counselor colleagues about how to handle legal problems, and at the same time it is inappropriate to consult with an attorney when making difficult clinical decisions. A counselor colleague might give you advice about a legal situation, but you cannot rely on the accuracy of that advice because your counselor colleagues are not lawyers. Likewise, a lawyer might give you advice about how to handle a difficult clinical issue, but lawyers are not educated as mental health professionals. If you ask attorneys about what to do in difficult clinical situations, they will focus on protecting you rather than on knowing what might be best for your client.

1-5 The Case of Fatima

Fatima is a counselor in a university counseling center who is in the process of planning for a month-long vacation. She will be leaving in 2 months. She has been having a series of conversations with her supervisor and several of her colleagues at the center about the proper way to handle her existing clients: when to tell them she is leaving; whether to refer them permanently to other counselors; whether to refer them temporarily to other counselors and take them back when she returns; whether to terminate with those who are nearing readiness; and what to do about those clients who have expressed suicidal ideation in the recent past. At the staff meeting today, the counseling center director announced that he had talked to the university attorney about the situation and that her advice was just to tell all of Fatima's clients that she will be gone for a month and to let them decide what to do in the interim. The director then said that the attorney had said that Fatima had no other legal obligations to the clients.

- To what degree should the university attorney's advice on this issue affect Fatima's decision making?
- Because the director told Fatima and the staff about the advice of the university attorney, how should Fatima interact with the director if she decides to do something different from what the attorney advised?

Discussion: It is good for counselors to get advice from attorneys regarding their legal obligations. However, a counselor's ethical obligations may go beyond what is required legally. Fatima will want to be careful not to offend the director, so perhaps she could thank the director for obtaining legal advice. Later and in private, she might share the ACA *Code of Ethics* with the director and point out some of the ethical obligations she wants to meet. She could then suggest that a good goal for the center would be to follow ethical and best practice guidelines, in addition to legal obligations, in order to provide the best possible service to clients.

When counselors face issues that require them to exercise their clinical judgment, particularly when there are no clear right or wrong responses, it is essential to consult with colleagues to the extent possible. In some situations, consultation might be impossible, such as when emergencies arise. When time does allow, however, consulting about clients provides a substantial protection to counselors whose clinical decisions are later challenged. The legal standard of care for counselors is that counselors must practice in a manner consistent with the way in which a reasonable, similarly educated counselor would practice under the same set of circumstances. By consulting with others, counselors can prove later that they indeed met the standard of care by doing what other, presumably reasonable, counselors advised or agreed on. If experts are available for consultation, it is wise to talk with them as well. Experts might include former university professors, counselors who are known for their expertise in a particular area, or counselors with extensive clinical experience.

It is impossible for counselors to know for certain whether they are making decisions that will protect their clients or others from harm. Because of this uncertainty, it is essential that counselors maintain current individual professional liability insurance policies at all times. Professional liability insurance policies are discussed in detail in Chapter 8.

Summary and Key Points

This introductory chapter has familiarized you with some key concepts that form the foundation for studying ethical, legal, and professional issues in counseling. Following are some of the main points made in this chapter.

The professional orientation content area of graduate training involves the study of professional identity, ethics, and law.

- *Morals, values*, and *ethics* are all interrelated terms, but they have different meanings as used in this text. The term *morality* refers to personal beliefs and how these beliefs affect your conduct, whereas *ethics* refers to professional behaviors. Counselors share certain professional *values*, and they hold their own unique personal values, which they must take care not to impose on clients.
- When confronted with a dilemma, counselors need to be able to distinguish whether they are dealing with a legal, an ethical, or a professional issue.
- A model of professional practice was presented as one way to conceptualize how multiple factors might fit together in a counselor's actual practice. The model contains internal and external building blocks that contribute to the development and maintenance of sound, professional practice.

Counselors should take their ethical responsibilities very seriously. Understanding ethics involves learning about its foundations, recognizing different approaches to ethical decision making, becoming familiar with codes of ethics, and developing ethical decision making skills.

- Theories of ethics have existed for centuries and provide a foundation for the ethical reasoning of contemporary professional counselors.
- Principle ethics, which is based on the moral principles of the helping professions, and virtue ethics, which is focused on traits of character, are two complementary ways of looking at the foundations of counselor ethics.
- Codes of ethics are vital resources in ethical decision making, but they cannot answer every question that might arise in actual practice. The ACA *Code of Ethics* (ACA, 2014) is the primary code on which counselors rely.
- Of critical importance to counselors is the development of ethical decision-making skills. Counselors are required to use an ethical decision-making model to reason through the ethical dilemmas confronting them.

Laws are agreed-on rules that set forth principles that allow people to live together in a society.

Laws dictate minimal standards of behavior, and ethics prescribes both minimal professional behaviors and ideal or aspirational behaviors.

- Most areas of counselor practice are affected by law.
- Counselors must know how to recognize legal issues and how to obtain legal advice.
- Counselors should seek advice from fellow mental health professionals when they have clinical or ethical questions and should seek advice from lawyers when they have legal questions.
- There is no substitute for professional judgment on difficult ethical or legal questions encountered by counselors, although many helpful resources exist.

Professional Identity of Counselors

2

LEARNING OBJECTIVES

2.1 Explain how the wellness model of mental health in counseling differs from the medical model.

2.2 Demonstrate an understanding of the difference between state licensure and national certification for counselors.

2.3 Explain how state statutes that license counselors vary so much in what is required to be licensed.

2.4 Demonstrate an understanding of the organizational structure of the American Counseling Association, including its divisions.

2.5 Identify the current and major legal and political issues facing the continued development of the counseling profession.

2.6 Indicate how counselors are similar to and different from other mental health professionals, including psychiatrists, psychologists, and social workers.

FOCUS QUESTIONS

1. How do you respond when your friends and relatives ask you what you are studying in graduate school?
2. How do you think the wellness model of mental health espoused by counselors is different from the illness model or medical model of mental health?
3. What are some of the major challenges facing the counseling profession today?
4. How do you think your experience today as a counseling graduate student is different from that of students who graduated pre–COVID-19?

Counselors are a relatively new professional group compared to such other mental health professionals who practice as psychologists, social workers, and psychiatrists. Potential consumers of mental health services often are unaware that counseling is a distinct profession that can be clearly distinguished from similar mental health professions. Even counselors themselves sometimes find it difficult to describe the distinctions. Thus, a vital professional task for counselors is to adopt a strong professional identity and to be able to articulate that unique identity to others (Gale & Austin, 2003; Healey & Hays, 2012; Smith, 2001).

Professional identity is a nebulous concept, but it is vital to the long-term success of a profession. Individuals who have a strong professional identity can easily explain the philosophy that underlies the activities of their professional group, describe the services that their profession renders to the public, describe the training programs that prepare them to practice their profession, explain their qualifications and

the credentials they possess, and articulate the similarities and differences between members of their own profession and members of other, similar groups. In addition, those with a strong professional identity feel a significant pride in being a member of their profession and can communicate this special sense of belonging to those with whom they interact.

A number of articles have appeared regarding counselor identity over the last few years. Counselor professional identity development has been discussed and studied from various perspectives (Burns & Cruikshanks, 2017; Cinotti, 2014; DeKruyf et al., 2013; Dollarhide et al., 2013; Field, 2017; Gibson, 2022; Healey & Hays, 2012; Hodges, 2011; Moss et al., 2014; Reiner et al., 2013; Woo et al., 2016). Scholars have recognized the importance of understanding counselor professional identity development for the success of the counseling profession.

In 2010, a definition of *counseling* was agreed upon by various organizations that represent the profession (Kaplan et al., 2014). This agreed-upon definition, which can be found on the American Counseling Association (2018a) website, is "Counseling is a professional relationship that empowers diverse individuals, families, and groups to accomplish mental health, wellness, education, and career goals." This is a simple definition meant for the general public. This chapter offers a much more thorough discussion of counseling, the counseling profession, and the professional identity of counselors.

2-1 The Case of Rivka

Rivka is a third-grade teacher who recently completed her master's degree in counseling. Because her school district employs counselors only at the high school level, she has assumed that she will have to move to another district to fulfill her goal of being an elementary school counselor. She is very excited when her principal calls to tell her that the school board is going to consider hiring either counselors or school social workers for the elementary schools. The principal asks her to attend the next school board meeting and speak about what an elementary school counselor does and how counselors differ from social workers. Rivka wants to give a very persuasive talk so that the board will decide to hire counselors and, she hopes, offer her one of the new positions. She asks you for advice on what to say and how to say it.

- What advice would you give Rivka, in terms of the information she should present?
- What arguments do you think would be most persuasive in influencing the board members to hire counselors?

Discussion: Rivka might gather all the information available from the American School Counselor Association (ASCA; www.schoolcounselor.org) regarding the unique role of school counselors in elementary schools. She should summarize this information for the school board members. She should also be prepared to explain the differences between the roles of school social workers and school counselors at the elementary school level.

The most influential information Rivka could present would be data showing that elementary school counselors make a difference in schools in terms of students' academic achievement; satisfaction of parents with the school; and satisfaction of parents, teachers, and administrators with elementary school counseling programs (Baker et al., 2021). Providing testimonials from students, parents, teachers, and principals in schools that have effective elementary school counseling programs might also be very effective.

We hope this chapter helps you clarify your professional identity as a counselor and helps you tell others about the profession of counseling. It also is intended to help you understand and appreciate the history of the counseling profession, the professional associations that serve counselors, graduate program accreditation, and the credentials available to counselors.

The Impact of the COVID-19 Pandemic on the Professional Identity of Counselors

When the counseling profession was established over a half-century ago, professional counselors could not imagine counseling clients when the counselor and the client were not in the same room. Counselors sincerely believed they had to by physically present with clients to communicate fully and effectively. They believed that clients and counselors had to not only hear each other, but that counselors also needed to see the client so they could assess body language and messages that are communicated without words. Counselor educators almost universally rejected the idea that counseling students could be educated without being physically together in classrooms with their professors. Contemporary counselors continue to appreciate the value of nonverbal communication and face-to-face human contact, but the COVID-19 pandemic has caused counselors to reassess whether face-to-face contact during counseling sessions and preparation of counselors is necessary or even preferable to the new options.

Although people were talking on the telephone during the 1950s, technology that allowed people to email, text, or see each other on video as they talked was unimaginable. When video technologies such as Zoom became available, many counselors continued to reject the idea that legitimate counseling sessions could take place on video or that counselors could be trained online. While counselors were debating these questions, the COVID-19 pandemic arrived, and the profession of counseling and the process of educating counselors were reimagined.

Contemporary counselors learned from the COVID-19 pandemic that effective counseling is possible using digital methods and that counselors can be appropriately trained online. Counselors and counselor educators have shifted from being suspicious of the use of video sessions in counseling or preparing counselors online to learning how to do both effectively and ethically.

PHILOSOPHY UNDERLYING THE COUNSELING PROFESSION

Counselors have a distinct belief system regarding the best way to help people resolve their emotional and personal issues and problems. This belief system provides the foundation for the professional identity of counselors. Basically, counselors share the following four beliefs regarding helping others with their mental health concerns:

1. The best perspective for assisting individuals in resolving their emotional and personal issues and problems is the wellness model of mental health.
2. Most of the issues and problems that individuals face in life are developmental in nature, and understanding the dynamics of human growth and development is essential to success as a helper.
3. Prevention and early intervention are far superior to remediation in dealing with personal and emotional problems.
4. The goal of counseling is to empower individual clients and client systems to resolve their own problems independently of mental health professionals and to teach them to identify and resolve problems autonomously in the future.

THE WELLNESS MODEL The first belief that counselors share is that the wellness model of mental health is the best perspective for helping people resolve their personal and emotional issues and problems (Ey et al., 2016; Leppma & Young, 2014; Mellin et al., 2011; Williams & Ramsey, 2017). Myers et al. (2000) developed a comprehensive model of wellness specific to counseling. Historically, the primary model used by other mental health professionals in the United States to address emotional problems was the medical or illness model, an approach created by physicians in caring for persons with physical illnesses. In the medical model, the helper identifies the illness presented by the person asking for assistance. The diagnosis of the illness is always the first step in helping. This perspective assumes that the client is diminished in some significant way. The goal of the professional helper is to return the help-seeker to the level of functioning enjoyed before the illness occurred. Once the illness has been isolated, the helper applies scientific principles in curing the illness. If the helper is successful and the illness is cured, the client then goes on about life. If another illness negatively affects the client's well-being, the client returns to the helper to be cured again.

Psychiatrists, who are physicians, are educated to approach mental health issues utilizing the medical model. Other mental health professions, including clinical psychology, psychiatric nursing, and clinical social work, came into existence when the medical model was prevalent, and these mental health professionals have their roots in this tradition as well.

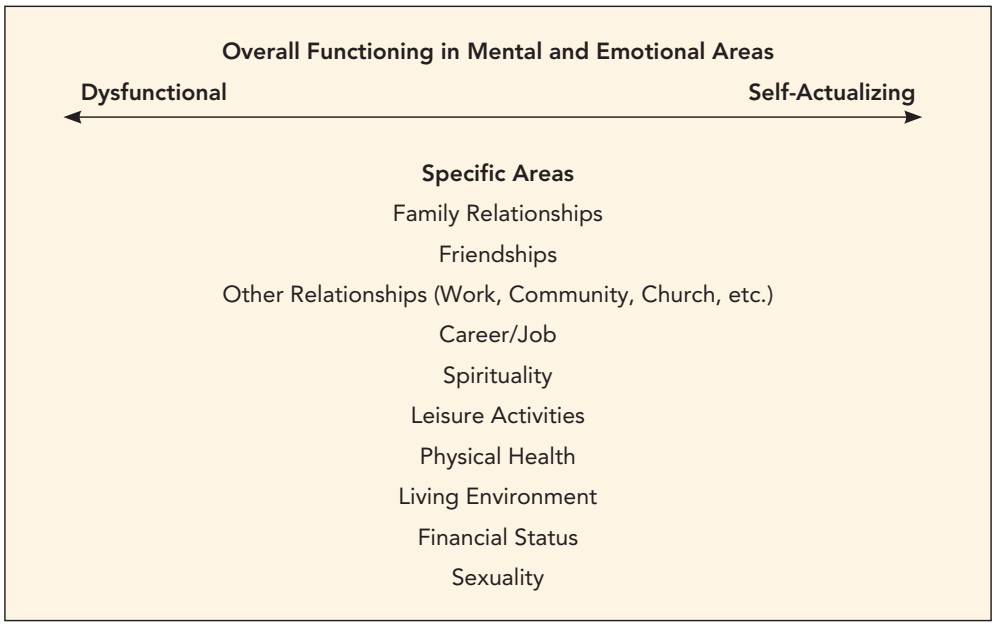
Counselors, on the other hand, belong to a newer profession with a different tradition. Counselors have adopted the wellness model of mental health as their perspective for helping people, and there is evidence that counseling from a wellness perspective is an effective method of helping clients (Myers & Sweeney, 2004; Tanigoshi et al., 2008). A meta-analysis completed by Weigold et al. (2020) supported the focus of the counseling profession on wellness. A construct known as *personal growth initiative* is an individual's active and intentional desire to grow in areas that are personally important. An analysis of research related to personal growth initiative suggested it can be increased through counseling and that having personal growth initiative increases success in counseling. In the wellness model, the goal is for each person to achieve positive mental health to the degree possible. From a wellness perspective, mental health is seen as occurring on a continuum (Smith, 2001). At one end of the scale are individuals who are very mentally healthy. Maslow (1968) described people who are fully functioning mentally and emotionally as *self-actualizing*. At the other extreme are persons who are dysfunctional because of mental health problems. Such people might include persons who do not have the capacity to respond to any kind of mental health treatment or intervention.

In addition to this general continuum of mental health, the wellness orientation also views mental health as including a number of scales of mental and emotional functioning (see Figure 2-1). These scales represent an individual's mental and emotional wellness in important areas of living. Counselors assess a client's functioning in each of these areas to determine where attention within counseling might best be focused to increase wellness. These areas include family relationships, friendships, other relationships (work, community, church, etc.), career/job, spirituality, leisure activities, physical health, living environment, financial status, and sexuality.

Counselors assess clients' current life situations and help determine which factors are interfering with the goal of helping clients reach their maximum potential. Many persons are limited by physical disabilities or environmental conditions that they cannot change. Keeping such limitations in mind, counselors assist their clients in becoming as autonomous and successful in their lives as possible.

Although the distinctions between the medical model and the wellness model can be clearly articulated, there is considerable overlap when they are put into practice. Many individual practitioners within the other mental health professions deviate from the illness model. In fact, evidence

FIGURE 2-1 The wellness orientation to mental health



that the medical profession has adopted many elements of the wellness model can be seen in current trends toward preventive medicine, consumer education, and patient rights. Increasingly, medical practitioners are coming to view patients as partners in their own health care, and this trend is also evident in the approaches of many psychiatrists, psychologists, and other mental health professionals. At the same time, today's counselors are educated to use the medical model of diagnosing mental disorders (the *DSM* system; American Psychiatric Association, 2022) and often render such diagnoses as a component of the services they provide.

Although wellness is a goal that counselors have for their clients, until recently few models existed to explain to counselors how to achieve wellness integration in their practices. Long et al. (2022) conducted a study in which 21 professional counselors were asked to identify the approaches they used to help their clients achieve wellness, and then used the information provided by the study participants to develop a model. In the model proposed by Long et al., counselors assess barriers to wellness that clients are experiencing and use counseling skills to implement wellness interventions for each particular client.

Within the counseling profession there is an increasing recognition of the importance of advocating for clients who face societal and institutional barriers that restrict their access to resources or inhibit their growth and development (Hunt et al., 2006; Ingersoll et al., 2004; Myers & Sweeney, 2004). Client advocacy has long been a tradition in social work practice, but until recently it has not been emphasized in the training of counselors, other than rehabilitation counselors. Mental health professionals who operate from the illness model practice in ways that are similar to the practices of mental health professionals who embrace the wellness model. For example, mental health professionals who espouse either model would most likely provide individual or group counseling services, spend time talking with clients, take clinical notes, and render a diagnosis of any mental disorders the person may have. Perhaps the primary differences between the two models are in the attitude of the professional toward the client and the focus of the professional's clinical attention. Counselors see the client as having both the potential to

make self-affirming choices and to succeed in living rather as than having an illness that needs to be remediated. In addition, the goal of counseling is to help the person accomplish wellness rather than to cure an illness. Hansen (2003, 2005, 2006, 2007, 2012) and Hansen et al. (2014) have written a series of articles that question many of the current practices and language used by counselors, suggesting that the counseling profession may be moving away from its foundational beliefs by classifying itself as a health care profession.

A DEVELOPMENTAL PERSPECTIVE A second belief that counselors share is that many personal and emotional issues can be understood within a developmental perspective. As people progress through the life span, they meet and must successfully address many personal challenges. Counselors believe that most of the problems people encounter are developmental in nature and therefore are natural and normal. Problems that some other mental health professionals might view as pathological, and that counselors would see as developmental, include the following:

- A 5-year-old crying as if in terror when his mother drops him off for the first time at his kindergarten class
- An 11-year-old girl who seems to be obsessed with boys
- A teenager vigorously defying their parents' directives
- A 19-year-old becoming seriously depressed after breaking up with their romantic partner
- A young mother becoming despondent soon after the birth of a child
- A 35-year-old beginning to drink so much they are getting into trouble after 15 years of social drinking
- A 40-year-old parent feeling worthless after their youngest child leaves home for college
- A 65-year-old professional person feeling very depressed as their retirement approaches
- An 80-year-old concerned that they are losing their mind because they are forgetting so much

By studying the developmental stages in life and understanding tasks that all individuals face during their lives, counselors can put many problems that clients experience into a perspective that views these problems as natural and normal. Even problems viewed as psychopathological by other mental health professionals, such as severe depression, substance addiction, or debilitating anxiety, could be seen as transitory issues that often plague people and that must be dealt with effectively if individuals are to continue living in a successful fashion.

PREVENTION AND EARLY INTERVENTION A third philosophical assumption of counselors is our preference for prevention rather than remediation of mental and emotional problems (McCormac, 2014; Nelson & Tarabochia, 2017; Wester & McKibben, 2016). When prevention is impossible, counselors strive toward early intervention instead of waiting until a problem has reached serious proportions.

A primary tool that counselors use to prevent emotional and mental problems is education. Counselors often practice their profession in the role of teacher, using psychoeducation as a tool. By alerting clients to potential future areas that might cause personal and emotional distress and preparing them to meet such challenges successfully, counselors prevent problems before they arise. Just a few examples of prevention activities are parenting education programs, anti-bullying groups for schoolchildren, career exploration groups, and pre-marriage counseling.

When the time for prevention has already passed and a client is experiencing personal or emotional problems, counselors prefer seeing clients early in the process. Counselors believe that counseling is for everyone, not just for individuals who have mental illnesses or emotional disorders. By providing services to individuals when they begin to experience potentially distressing events

in their lives, counselors hope to intervene early and thereby prevent problems from escalating. For instance, counselors would prefer to see a client who is beginning to have feelings of depression rather than someone who could be diagnosed as having an episode of severe depression, and counselors would encourage couples who are beginning to experience problems in their relationship to seek counseling rather than wait for their problems to escalate into serious marital discord.

EMPOWERMENT OF CLIENTS The fourth belief that counselors share regarding the helping process is that the goal of counseling is to empower clients to problem-solve independently (Chronister & McWhirter, 2003; Dailey et al., 2014a; Savage et al., 2005). Through teaching clients appropriate problem-solving strategies and increasing their self-understanding, counselors hope that clients will not need assistance in living their lives in the future. Realizing that individuals often need only transitory help, counselors also try to communicate to clients that asking for and receiving help is not a sign of mental or emotional weakness but, instead, is often a healthy response to life's challenges.

It is quite easy for individuals to become dependent on those who provide help to them. Some systems of mental health care seem to encourage a pattern of lifelong dependence. Counselors recognize this problem and encourage clients to assume responsibility for their lives and learn to live in a manner that allows them autonomy and independence as those concepts are understood within the clients' cultures. The ACA *Code of Ethics* (2014) in the preamble states that one of the fundamental principles of ethical behavior of counselors is "*autonomy*, or fostering the right to control the direction of one's life." Although some people may need assistance throughout their lives because of a physical or mental disability, all clients are helped to become as independent as their circumstances will allow. Counselors do not present themselves as experts in mental health who must be consulted when problems arise. Rather, counselors communicate the belief that clients are capable of developing the skills they need for independent living and wellness.

Counseling Services

To achieve a strong professional identity as a counselor, it is essential that you understand the philosophy of helping that we have just described. In addition, you need to be knowledgeable about the kinds of services that counselors offer to the public, even though the services that counselors provide do not define the profession. Actually, counselors engage in many of the same activities as other mental health professionals do. The key to a strong professional identity is the philosophy underlying the services, not the services being rendered.

The basic service that counselors provide is counseling—for individuals, couples, families, and groups. All other mental health professionals counsel patients and clients, too. A major difference between counselors and other mental health professionals who counsel is that counseling is the *primary* professional service that counselors provide. By contrast, the primary service that psychiatrists provide is the diagnosis and medical treatment of mental disorders as a physician; psychologists primarily provide assessment and research; psychiatric nurses provide management of mental health care within a hospital setting; and social workers primarily link clients to community resources. For mental health professionals other than counselors, counseling may be a secondary or ancillary service.

In addition, many other professionals outside the mental health field call the service that they provide to clients *counseling*. For example, attorneys provide legal counseling, accountants offer financial counseling, and physicians offer counseling to their patients regarding their physical health. Counselors, in contrast to these non–mental health professionals, provide mental health counseling services to clients.

In conjunction with counseling, counselors also perform a number of other professional services. These services include assessment, teaching, diagnosis, case management, and advocacy. Despite performing these other duties, the primary service provided by counselors is mental health counseling.

Counselors have many job titles, roles, and duties in their professional positions. To develop a strong professional identity, you must know when you are functioning as a counselor and when you are performing other roles in your professional position. You must also be able to identify the types of professional services that counselors render to the public.

PREPARING AND CREDENTIALING COUNSELORS

Ask lawyers what they were taught in law school, physicians what they were taught in medical school, and engineers what they were taught in engineering school, and they can easily and clearly describe the courses they took, the practical components of their educational experience, and the topics covered in their preparation programs. To develop a strong professional identity, it is vital that you are also able to describe your training program components. By describing the educational programs that prepare counselors, you summarize the essential nature of counseling as a profession and emphasize the basic knowledge required for becoming a counselor.

Counselor Preparation Programs

An important aspect of counselor training is that the profession considers individuals to be professionals after they have completed a master's degree. Although a master's degree is the required professional degree in both psychiatric nursing and clinical social work, psychiatry requires a medical degree and a residency in psychiatry, and psychology requires a doctoral degree for the achievement of professional status. One difference between counseling psychology and the profession of counseling is that counseling psychologists must hold doctorates to be considered professionals, whereas counselors are considered professionals at the master's-degree level.

In addition, counseling skills are the primary focus of the counseling master's-degree training programs. Although other subjects are taught, all the training emphasizes competency in the areas of individual and group counseling. The other courses required in master's-degree programs, such as research, assessment, multicultural counseling, and career counseling, are meant to strengthen the counseling skills of graduate students.

Counseling graduate programs generally are located in colleges of education within universities. The profession of counseling has its foundations in pedagogy and psychology, particularly in counseling psychology and human growth and development. All these fields have their roots in colleges of education, which is why counseling graduate programs are usually located there. Some counselor educators and counselors are concerned that being located in a college of education implies that counselors are being prepared to function only in educational settings, such as schools or colleges. They believe that the general public's perception is negatively affected when they think of all counselors as being prepared and practicing in the same way that school counselors were prepared and practiced at the beginning of the school counseling movement in the 1950s (Baker et al., 2021; Levy & Lemberger-Truelove, 2021; Morris et al., 2021; Zyromski et al., 2018). The reality is that the counseling process is educational in nature, and although counselors do not educate in formal classrooms like traditional teachers do, counselors do educate their clients. Being located in colleges of education strengthens counseling graduate programs because the colleges emphasize pedagogy, human growth and development, applied research, and field placements as essential components of learning skills.

The counseling profession created the Council for Accreditation of Counseling and Related Educational Programs (CACREP) to set national standards for the preparation of master's- and doctoral-level counselors (CACREP, 2024; Urofsky, 2013). CACREP was begun during the 1960s under the leadership of Robert O. Stripling, a counselor–educator at the University of Florida. CACREP began as a project of the Association of Counselor Education and Supervision (ACES), a division of the ACA, and now functions as a separate corporation.

CACREP has established standards for the preparation of counselors (CACREP, 2024). These standards require a minimum of semester hours in a master's-degree program that prepares counselors. In addition, the programs must include instruction in the following content areas: (a) professional counseling orientation and ethical practice, (b) social and cultural identities and experiences, (c) lifespan development, (d) career development, (e) counseling practice and relationships, (f) group counseling and group work, (g) assessment and diagnostic processes, and (h) research and program evaluation. CACREP requires that students complete a 100-hour practicum and a 600-hour internship. The CACREP standards also require that program faculty be counselors, that programs have a laboratory for training, and that a number of other requirements for quality instruction be met (Bobby, 2013; Even & Robinson, 2013; Lee, 2013).

Not all master's-degree programs in counseling are accredited by CACREP. Many programs that are not yet accredited have patterned their master's-degree programs after the standards developed by CACREP. Although not all the counseling graduate-degree programs in the United States are accredited by CACREP, it is accurate to say that CACREP standards have been accepted by the profession as the model curriculum for preparing master's- and doctoral-level counselors (Lawson, 2016). Because graduation from a CACREP-accredited program is one type of credential that a counselor might possess, CACREP accreditation will be discussed further in the next section.

Credentialing

One of the greatest challenges faced by those entering the counseling profession is to understand counselor credentialing. Credentialed individuals possess some type of indicator that they are legitimate professionals. Credentialing comes in many forms, which is the basic reason people are so confused by it. In addition, some credentials are essential to the practice of the profession; others are desirable but not necessary; and still others are of questionable value or even worthless. This section contains a discussion of credentials and should assist you in conceptualizing and understanding counselor credentials.

A major problem that causes confusion among counselors and the public is that terminology is not consistent within the area of credentialing. The major types of counselor credentials are degree, state license, state agency certification, national voluntary certification, and program accreditation.

DEGREE The most obvious credential that counselors hold is the master's degree. Other degrees, such as educational specialist or doctoral degrees, are credentials as well. Counselors hold a variety of master's-degree titles. A legitimate counselor might hold a Master of Education (MEd), Master of Arts (MA), Master of Science (MS), Master of Divinity (MDiv), or another, more unusual, master's-degree title such as Master of Counseling (MCoun). Most of the titles

of master's degrees given in various universities were decided long ago and generally have little to do with any differences in the actual program content. The degree title in no way indicates whether a degree program requires a specified number of credits, includes a thesis, or has other specific requirements. In many universities, degree titles are decided politically. For example, many counseling master's-degree programs award MEd degrees because the degree programs are offered within university colleges of education.

Although the basic credential for a professional counselor is a master's degree in counseling, many counselors hold higher degrees, including educational specialist's degrees, certificates of advanced study, and doctoral degrees. Doctoral degrees in counseling usually are either the Doctor of Philosophy (PhD) or the Doctor of Education (EdD), although some counseling doctoral programs might have titles such as Doctor of Arts (DA) or Doctor of Divinity (DDiv).

STATE LICENSE In most states, a license issued by the state is required before a person is allowed to practice counseling in that state (Bergman, 2013; Morgen et al., 2012). This licensure affects private practice counseling most directly because most licensure statutes state that counselors who practice in many other settings are exempt from the licensure requirement. For example, in most states, counselors who practice in local, state, or federal agencies; in nonprofit corporations; or in schools or other educational institutions are exempt from the licensure requirement. Often when professions are first licensed, many exemptions are granted to the licensure requirement. This occurs because of a concern that, if licensure were required for all members of that profession, agencies that traditionally pay less for the professionals' services would not be able to attract employees. For example, when physicians were first licensed by states, physicians employed in many settings were exempt from licensure. Over the years, these licensure exemptions have been removed for physicians, and now almost all physicians, no matter where they practice, must have a state license. The same removal of exemptions will most likely occur over time in the counseling field.

Unfortunately, counseling licensure statutes have various titles. In some states, counselors credentialed by the state are called *licensed*, and in other states they are referred to as *certified*. In some states, they have even been called *registered*.

In the more general arena of state regulation of professions, the terms *licensed, certified*, and *registered* have separate and specific meanings. A licensure law is referred to as a *practice law*, which means that the professional must be licensed to practice that profession in that state. A certification law is what is known as a *title law*, which means that a person must be certified to use the title of *certified professional* but that the practice of that profession is not regulated by the state. Finally, a *registration law* was intended to mean that a professional had to *register* with the state but that no credentials would be required for registration. These terms, however, have been used interchangeably from state to state and from profession to profession. For example, registered nurses in almost all states in reality are *licensed* even though their title indicates they are *registered*. In addition, most current state registration laws do require that registered persons have some type of credentials before they are added to the state registry.

Currently, in some states licensed counselors actually function under title laws, and some certified counselors, in reality, are operating under practice laws. As an individual counselor who wants to practice in a particular state, you should ascertain exactly what the state credential (whether a license, a certificate, or a registry) entitles the credentialed person to do. You do not have to fully understand the complexity of all state regulatory laws to understand what the particular law means in your state. The American Association of State Counseling Boards (AASCB, 2023) and the American Counseling Association (ACA, 2023) provide contact information for

state licensure boards (AASCB, 2023). In addition, the National Board for Certified Counselors (NBCC) provides contact and testing information for state counselor licensure boards (NBCC, 2023). All 50 states, the District of Columbia, Puerto Rico, and Guam now have laws that regulate the profession of counseling.

To add to the confusion regarding state counselor licensure laws, states have given several titles to counselors who are licensed. The most common title is that of *professional counselor*. However, other states use titles such as *mental health counselor* or *clinical counselor*. The individuals who proposed the counselor licensing laws in each state decided which title to use. In some cases, titles were chosen to satisfy political compromises that had to be made to obtain legislative support for the bills.

STATE AGENCY CERTIFICATION As the term is used here, *state agency certification* is different from state licensure. It was explained previously that counselors in some states are certified rather than licensed. However, in this section, the term *certification* is used to refer to the process whereby official agencies of the state certify individuals as qualified to hold certain state jobs. In most states, this agency process is referred to as *certification*, although some call it *licensure*.

The most obvious state agency certification process is for school counselors. In each state the department of education determines which counselors may be certified as school counselors. To maintain their school accreditation, schools in that state must hire certified school counselors, just as they must hire only certified school teachers. The requirements for being certified as a school counselor vary widely from state to state. In some states, school counselors must first be certified and experienced school teachers before they can become certified as school counselors, although this teacher certification requirement is being eliminated in most states. The requirements for state school counselor certification should be obtained from the state department of education in the state in which you wish to practice. The American Counseling Association (2023) and the American School Counselor Association (2023) provide information about school counselor certification in all states.

Another area in which counselors are certified by the state is in addictions counseling (Morgen et al., 2012). Most states have various levels of certified addictions counselors, sometimes including counselors who hold less than a bachelor's degree. State agencies that provide addictions treatment services are required to hire only certified counselors who meet state requirements for certification. Information regarding state requirements for addictions counselors can be obtained from NAADAC: The Association for Addiction Professionals (NAADAC, 2023).

Rehabilitation counseling is another specialty area that involves varying state requirements. Although state rehabilitation agencies do not certify vocational rehabilitation counselors, they have established minimum requirements for rehabilitation counselors that vary from state to state. Information regarding the requirements for being hired as a vocational rehabilitation counselor can be obtained from each state's rehabilitation agency.

NATIONAL VOLUNTARY CERTIFICATION The two national certification agencies of the counseling profession are the National Board for Certified Counselors (2023) and the Commission on Rehabilitation Counselor Certification (CRCC, 2023). In 2023, NBCC certified more than 70,000 counselors in more than 40 countries (NBCC, 2023). In 2023, CRCC had over 15,000 certified rehabilitation counselors (CRCC, 2023). National certification is voluntary in that there is no governmental requirement that a counselor be certified for private practice. Even though a counselor may be required to be licensed in a state to practice counseling, there is no legal need for that same counselor to be certified by NBCC or CRCC. Counselors choose to become nationally certified to demonstrate

they have met the highest national standards developed by their profession. National certification is sometimes used as a job prerequisite as well, especially in the field of rehabilitation counseling.

The counseling profession created NBCC in 1982 as the national agency to certify counselors who had met the minimum requirements of the profession (NBCC, 2023). A National Certified Counselor (NCC) is an individual who has met the requirements set forth by NBCC. NBCC now functions as an independent corporation with its own board of directors. It was important for the profession to create NBCC so that the profession, rather than each individual state legislature, could determine the minimum requirements for being a professional counselor. In the same vein, it is important for counselors to support NBCC by becoming NCCs as soon as they are qualified to do so. In addition, counselors who hire other counselors should consider indicating that NCCs are preferred.

To become an NCC, a counselor must complete a master's degree in counseling, have 2 years of post-master's experience, and pass the National Counselor Examination. The 2 years of post-master's experience is waived for graduates of CACREP-accredited programs.

Every profession has national voluntary certification of specialists within that profession. NBCC has created specialty certification for the counseling profession. Although some believe that states should license counselors as specialists (Cottone, 1985; Emener & Cottone, 1989), we believe states should license counselors generally and NBCC should certify specialists (Remley, 1995). Our position is that the role NBCC plays in certifying counselors as specialists within their chosen areas is vital to the success of the counseling profession. We believe that specialty designations should be voluntary and should be offered by a national body such as NBCC.

To become certified as a specialist by NBCC, a counselor first must be an NCC. Currently, NBCC offers specialty certification in addictions counseling, mental health counseling, and school counseling. The specific requirements for becoming specialty-certified in one or more of these fields can be obtained by contacting NBCC (2023). In addition, an affiliate of NBCC, the Center for Credentialing & Education (CCE, 2023) offers additional specialty certifications, including Approved Clinical Supervisor (ACS), Board Certified–TeleMental Health Provider (BC-TMH), Global Career Development Facilitator (GCDF), Board Certified Coach (BCC), and Human Services–Board Certified Practitioner (HS-BCP).

Rehabilitation counseling specialists began certifying rehabilitation counselors before NBCC was formed. As a result, that specialty certification is offered by a separate certifying agency, the Commission on Rehabilitation Counselor Certification (CRCC, 2023).

In addition to NBCC and CRCC, many other national voluntary counselor certifications are available to counselors. Some of these certifications have high standards and are multidisciplinary in nature. Others are simply designed to make money for the individuals who created them. If you are interested in a national certification for counselors offered by a group other than NBCC or CRCC, you should investigate the group's reputation among members of the counseling profession, the organizational structure of the group, and the general legitimacy of the credential.

The NBCC Foundation (2023), through the Global Capacity Building program, is providing leadership in educating counselors about counseling in other countries and is helping to advance mental health care worldwide. The program offers international institutes to counselors, helps countries establish counselor certification, and provides Mental Health Facilitator (MHF) training.

Program Accreditation

The training of counselors was summarized previously in this chapter. The standards for preparing counselors that have been agreed upon by the counseling profession are included in the accreditation standards of CACREP, also detailed in the previous section. The Council on Rehabilitation Education (CORE) formerly accredited master's-degree programs in the specialty area of

rehabilitation counseling. CORE and CACREP have now merged, and CACREP accredits rehabilitation counseling programs, along with other specialty counseling programs.

A counselor's graduation from a CACREP-accredited counseling graduate program is a credential. Although many competent counselors are graduates of non-CACREP-accredited programs, the CACREP credential does distinguish counselors as having completed a preparation program that meets the standards of excellence for the profession. Counselors can indicate on their résumés that they are graduates of CACREP-accredited programs. Many job announcements now indicate that CACREP-accredited program graduates are required or preferred.

Only master's and doctoral-degree programs offered by regionally accredited universities are listed in the directory. Nonaccredited universities also offer graduate degrees in counseling. However, individuals who graduate from these nonaccredited university degree programs often are unable to have their degrees recognized for the purposes of licensure, certification, or employment.

Accreditation of the university that offers degree programs in counseling, therefore, is essential for recognition. In addition to university accreditation, separate accreditation of counseling programs is important. Many state licensure statutes for counselors require the same preparation courses and field experiences as CACREP requires. In addition, NBCC has essentially the same preparation standards as CACREP.

CACREP (2024) accredits master's-degree programs in addictions; career; clinical mental health; clinical rehabilitation counseling; college counseling and student affairs; marriage, couple, and family counseling; rehabilitation counseling; and school counseling and doctoral programs in counselor education and supervision.

CACREP reflects the minimum preparation program standards that have been agreed upon by the profession for master's- and doctoral-degree programs in counseling. If you want to determine whether programs that are not accredited by CACREP reflect these minimum standards, you could compare the degree requirements of those programs to the CACREP standards.

In established professions such as medicine, law, and even psychology, all preparation programs are either accredited by their professional accreditation bodies or are actively pursuing accreditation. Graduates of nonaccredited programs in those professions are unable to obtain state licenses to practice or are unable to secure desirable jobs (LaFountain & Baer, 2001). It is possible that, eventually, all master's- and doctoral-degree programs in counseling will be CACREP-accredited. Counseling graduate programs that do not have CACREP accreditation will need to seek and obtain CACREP accreditation or will cease to exist because students will not want to attend programs that are not CACREP-accredited, a change that is being resisted by some within the counseling profession who believe that states have begun to add unreasonable requirements for counselor licensure. Ohio has recently passed a law that requires all future counselors who are licensed in that state to have degrees from CACREP-accredited programs (Bray, 2013).

Ethical Standards Related to Credentialing

Professional counselors are ethically required to present their credentials accurately. According to the ACA *Code of Ethics* (2014), counselors must claim or imply only professional qualifications actually completed and correct any known misrepresentations of their qualifications by others. They must clearly distinguish between paid and volunteer work experience and accurately describe their continuing education and specialized training (§C.4.a.). With respect to degrees held, counselors may advertise only the highest degree earned that is in counseling or a closely related field (§C.4.d.). Some counselors hold a master's degree in counseling and a doctoral degree in another field.

As you acquire various professional credentials, you certainly will want to advertise them. Your professional association's code of ethics imposes few restrictions on advertising (§C.3.), and those are limited to restrictions that can be clearly justified to protect the public. Nonetheless, you must know precisely what limits are contained in the code. In some circumstances, however, it is difficult to determine what the ACA *Code of Ethics* allows.

2-2 The Case of Kevin

Kevin earned his master's degree in counseling 8 years ago and has been working as a staff counselor in a psychiatric hospital since he graduated. In addition to this employment, he opened a part-time private practice when he became licensed as a professional counselor. Kevin recently completed his doctorate in health care administration from a nontraditional distance-learning university in California. He is now focused on his goals of moving into an administrative position within the hospital as well as building up his private practice in case an administrative job does not become available. He has decided to update the business cards and brochures he uses in his private practice, and he has changed the wording to present himself as "Kevin Smith, PhD, MS, LPC, Professional Member of the American Counseling Association."

- Do you see anything wrong with the wording on Kevin's cards?
- Do you believe a doctorate in health care administration could be considered a doctorate in counseling or a closely related field?

Discussion: All of Kevin's credentials on his card are directly related to counseling except perhaps his PhD, which is in health care administration. If his doctorate was earned from a university that was not accredited by one of the regional accrediting bodies, then the legitimacy of his degree would be questionable. He must be careful not to mislead the public into believing that he has a legitimate counseling doctoral degree. Whether the university from which he earned his degree is accredited by a regional accrediting body is not known. Many nontraditional distance-learning universities do hold regional body accreditation.

According to the ACA *Code of Ethics* (2014), "Counselors do not imply doctoral-level competence when possessing a master's degree in counseling or a related field by referring to themselves as *Dr.* in a counseling context when their doctorate is not in counseling or a related field" (§C.4.d.). There are arguments for and against recognizing a PhD in health care administration as a doctorate in a field closely related to counseling. On one hand, counseling is a form of health care, and a specialty within health care could easily be mental health care administration. On the other hand, general health care administration is not a mental health field. Perhaps the best way for Kevin to ensure that he is acting ethically, even though it might be a bit awkward, would be for him to specify on his business cards that his PhD degree is in health care administration and that his MS degree is in counseling.

Basically, advertising must be accurate and must not be false, misleading, deceptive, or fraudulent (ACA, 2014, §C.3.a.). When you become aware of any misrepresentation of your credentials that might be made by others, you are responsible for making reasonable efforts to correct the misinformation (§C.3.c.). Because many counselors hold multiple credentials, their business cards and advertising brochures can sometimes resemble a kind of alphabet soup. A counselor's degrees, licenses, certifications, and professional memberships might include, for instance, MS (Master of

Science) from a CACREP-accredited counseling program, LPC (Licensed Professional Counselor), NCC (National Certified Counselor), CCMHC (Certified Clinical Mental Health Counselor), CADAC (Certified Alcohol and Drug Abuse Counselor), and ACA Professional Member. It is important for counselors to make reasonable efforts to ensure that the *meaning* of these credentials is clear to potential consumers of their services.

In addition, as a credentialed counselor, you must be careful not to attribute more to your credentials than the credentials actually represent (§C.4.a.), and you should not imply that other counselors are not qualified because they do not possess the same credentials as you do or because they lack certain credentials (§§C.4.a. and D.1.a.). For example, it would be inappropriate for a Certified Alcohol and Drug Abuse Counselor to say or infer that a mental health professional had to have that credential in order to be competent to counsel clients regarding addiction or dependency problems. Also, it would not be professional to denigrate the preparation of licensed psychologists by inferring that their degree programs have prepared them less adequately for mental health practice than have the degree programs of licensed professional counselors. The *Code* states, "Counselors acknowledge the expertise of other professional groups and are respectful of their practices" (§D.1.a.).

THE ORIGINS AND EVOLUTION OF THE COUNSELING PROFESSION

History is in the eye of the beholder. In other words, people view history from their own personal perspectives. Several authors have attempted to summarize the history of the counseling profession, and each summary is different (Bradley & Cox, 2001; Field, 2017; Gibson & Mitchell, 2008; Hershenson et al., 1996; Lawson, 2016; Sweeney, 2001). Various authors have emphasized different events and have given their own interpretations to facts that have led them to various conclusions. The following summary represents a brief view of the history of the counseling profession that recognizes the input of others and adds our perspectives based on our observations as leaders and members of the profession.

While the psychology profession was establishing the doctoral level as the requirement for professional status and counseling psychology was developing as a specialty area within it, historical events were leading to the rapid development of school counseling programs and vocational rehabilitation counseling. Eventually, changes within counseling psychology, the school counseling movement, and federal funding of vocational rehabilitation counseling led to the emergence of the new profession of counseling.

Counseling Psychology

The counseling profession basically shares its history with the emergence of counseling psychology as a specialty within the psychology profession (Goodyear, 2000). Early leaders in the specialty of counseling psychology distinguished themselves from other psychologists by declaring that they were interested in the normal, healthy development of human beings rather than in illness and psychopathology. In counseling psychology, the focus was on developmental stages in people's lives and career concerns. World War I and the work of Frank Parsons on career classifications provided the impetus for the development of counseling psychology as a separate specialty within psychology. Parsons created the concept in the United States that citizens should be assisted in selecting careers that would be rewarding and that matched their abilities and interests (Gibson & Mitchell, 2008).

Although the field of psychology has offered doctoral degrees for quite some time, at the beginning of its political movement to become a profession, psychology recognized individuals with master's degrees as professional psychologists. Several decades ago, the American Psychological

Association (APA) declared that, in the future, only psychologists who held doctoral degrees would be recognized as professionals. The profession decided to continue to recognize all existing psychologists who held master's degrees and allow them to practice but, in the future, to allow only individuals who held doctoral degrees in psychology into the profession. Licensure laws in psychology throughout the United States were changed to reflect this new political position.

Despite APA's declaration that only psychologists at the doctoral level would be recognized as professionals, universities throughout the United States continue even today to offer master's degrees in clinical and counseling psychology. Essentially, these master's-degree programs in psychology have produced and are continuing to produce graduates who have limited or no professional recognition in psychology. Graduates of these master's-degree programs most often seek and obtain state licensure as counselors. When graduates of master's-degree programs in clinical or counseling psychology become counselors through the process of state counselor licensure, the professional identities of both psychologists and counselors become blurred, and the public becomes more confused about the differences between the two professions. In addition, a few licensure boards have adopted rules in the last few years that do not recognize master's degrees in psychology as acceptable for licensure as professional counselors.

School Counseling

When the Russians launched the first spacecraft, *Sputnik*, in 1957, politicians in the United States feared that, because the Russians had exceeded U.S. technology and beat the United States in the *race to space*, they might overpower the country politically as well. In response to this fear, the U.S. Congress created and funded substantial programs to encourage young people to seek careers in technical and scientific fields. Part of this effort included placing counselors in high schools to channel students into math and science courses. Throughout the United States, universities created summer institutes in which high school teachers were given basic courses that led to their placement in high schools as guidance counselors. In most instances, high school teachers were given two or three courses in *guidance* or *counseling*, which then allowed them to be certified as school counselors and assume guidance counselor positions within schools. Because the primary purpose of this effort was to encourage students to take math and science courses, it did not seem necessary for counselors to be prepared beyond the training provided in these summer institutes.

In a very short time, school accreditation groups required that high schools have guidance counselors to receive or continue their accreditation. Today, almost all middle and high school accreditation agencies require that these schools have counselors, and in some areas of the country, elementary schools are required to have counselors as well. Almost all states now require that certified school counselors have received the master's degree and have completed specified courses and an internship.

Vocational Rehabilitation Counseling

In the 1950s, it was recognized in the United States that citizens with physical and mental disabilities were not being given the help they needed to become productive members of society. As a result, legislation was passed that provided counseling and educational resources that were meant to rehabilitate persons with disabilities so that they could function as autonomously as possible.

A major component of this rehabilitation legislation was funding to prepare counselors to help persons with disabilities evaluate their disabilities, make plans to work, and find satisfactory employment. Because of this funding, master's-degree programs in rehabilitation counseling were developed, and existing programs were expanded. State rehabilitation agencies created positions in rehabilitation case management and counseling for the graduates of these programs.

In summary, the dynamics around the creation of the specialty of counseling psychology, the decision within the psychology profession that professionals would be recognized only at the doctoral level, the emergence of school counseling, and the funding of vocational rehabilitation counseling programs led to the creation of counseling as a separate profession. Clearly, the origins of the profession lie in the convergence of several disparate forces rather than in a single event.

Counseling as a New Profession

An entire area of scholarly inquiry in the field of sociology is devoted to studying the emergence of new professions. Scholars within this field have identified the essential factors that define what a profession is and what steps a group must go through to establish itself as a legitimate, separate, and unique profession. In relation to other professional groups in general and to mental health professional groups in particular, counseling is a relatively new professional entity. To understand some of the difficult problems facing the counseling profession today, it is important for counselors to compare counseling to other, older mental health professions and to ascertain where we are in the process of becoming a fully recognized profession.

Toren (1969) defined a fully developed profession as one that has the following traits: a body of theoretical knowledge, members who possess competence in the application of this knowledge, and a code of ethics focused on clients' well-being.

A different concept of a profession has been offered by Hughes (1965). He said that, compared to nonprofessions, emerging professions demand more independence, more recognition, a higher status, a clearer distinction between those inside and those outside the profession, and more autonomy in choosing colleagues.

Six criteria for determining professional maturity have been described by Nugent (1981). According to Nugent, a mature profession and its members have the following characteristics:

- They can clearly define their role and have a defined scope of practice.
- They offer unique services (do something that members of no other profession can do).
- They have special knowledge and skills (are specifically trained for the profession).
- They have an explicit code of ethics.
- They have the legal right to offer the service (have obtained a monopoly, through licensure or certification, over the right to provide the services).
- They have the ability to monitor the practice of the profession (the profession can police itself).

By considering these traits, conceptualizations, and definitions of a mature profession, it is possible to see that the field of counseling meets the criteria, although to different degrees. Certainly, counselors have a well-established code of ethics and have achieved societal recognition through state licensure. Areas that need to be strengthened include identifying services that are uniquely offered by counselors, developing a body of knowledge that belongs specifically to counseling and does not borrow so heavily from psychology, and achieving a higher status within society.

Steps in Becoming a Profession

According to sociologists who have studied emerging professions within society (Abbott, 1988; Caplow, 1966; Dugan, 1965; Etzioni, 1969; Friedson, 1983; Hughes, 1965; Scott, 1969; Toren, 1969), professions must go through a developmental process if they are to be successful in establishing themselves as fully recognized professions.

Caplow (1966) maintained that the steps whereby a group achieves professional status are quite definite, and that the sequence of steps is explicit. The first three steps toward professionalization are

(a) forming associations, (b) changing names to reduce identification with the previous occupational status, and (c) developing a code of ethics. Certainly, the counseling profession has accomplished these three steps. First, ACA was chartered in 1952 from three long-standing subgroups. Second, the association's name was changed from the American Personnel and Guidance Association (APGA) to the American Association for Counseling and Development (AACD) and then to the American Counseling Association (ACA). Each change reduced the identification with words such as *guidance, student personnel*, and *student development* and embraced more fully the word *counseling*. In addition, ACA has had a code of ethics since 1961.

According to Caplow (1966), the fourth step in accomplishing the goal of becoming a profession is prolonged political agitation. Counselors have been involved in substantial political agitation since the first counselor licensure law was passed in Virginia in 1976. That political activity has been continued as similar licensure laws have been passed in state after state, until licensure was gained in all 50 states. In addition, counselors have lobbied state legislators to mandate school counseling programs, to include counselors as providers in health care programs, and to ensure that the professional services of counselors are reimbursed by health care insurance companies. Clearly, counselors have engaged in Caplow's prolonged political agitation, and this process is continuing today.

Counselors have been fighting hard and making many changes to overcome the deficiencies that have hindered the development of the profession. Some of the changes that have taken place in the last 20 to 30 years in the field of counseling include the following: The length of most training programs has increased from 30 to 48 and then to 60 semester hours; efforts have been made to improve the professional status of counselors through credentialing and legislation; laws are continually passed in states granting privileged communication to interactions between counselors and their clients; the body of knowledge is being increased through scholarly publications specifically in counseling, as distinguished from psychology; federal programs such as Tricare and Medicare have begun to recognize counselors as mental health providers; and counselor licensure laws have brought autonomy from being supervised by others to practice counseling (Fullen et al., 2020a; Fullen et al., 2021; National Board for Certified Counselors, 2023; Westcott et al., 2022). Evidence suggests that counseling is a recognized profession in the United States.

Progress Toward Professionalization

It is discouraging for graduate students preparing to enter the field to realize that counseling, despite its progress, still has some way to go before achieving recognition on a par with other mental health professions within our society. On the other hand, it is encouraging to note that counseling is making progress toward recognition as a profession at a relatively fast rate when compared to professionalization efforts in other fields. For example, scholars within psychology wrote less than 50 years ago in one of their own profession's journals that they doubted that psychology could validly claim to be a profession (Phillips, 1982). The first law establishing psychologists as independent health practitioners was passed in 1968 in New Jersey (Dorken & Webb, 1980), and some states still do not require a license to practice psychology. In comparison, the first counselor licensure bill was passed in Virginia in 1976, and all 50 states, the District of Columbia, and other U.S. jurisdictions have already passed licensure bills for counselors.

PROFESSIONAL ASSOCIATIONS OF COUNSELORS

Professional associations are formed by professional groups for many reasons. First, associations provide a forum for professionals to gather to discuss issues and problems that exist within the profession. An association allows the members of a profession to address issues as a group

rather than work independently on behalf of the profession. Second, professional associations provide leadership by speaking for the profession on critical legislative issues that affect the profession at all levels of government (Li & Peters, 2022; McKibben et al., 2017). Professional associations also provide continuing education for their members. Earning a master's degree in counseling is only the beginning of the learning process for counselors. As new information becomes available, counselors must attend conferences and workshops, in person and virtually, to update their skills and expertise. Professional associations provide such learning opportunities and publish scholarly journals, texts, monographs, and other media resources for the continuing education of their members. In addition, professional associations publish and enforce a code of ethics for their members. Important hallmarks for professionals are having a code of ethics and policing themselves.

American Counseling Association

The American Counseling Association (ACA, 2023c) has many divisions and state branches that represent the counseling profession and serve counselors in various ways.

You should join ACA when you begin your graduate-degree program in counseling and maintain active membership throughout your career as a counselor. Students receive discounted membership. All members, including students, receive a subscription to the counseling professional journal, the *Journal of Counseling & Development*. In addition, members receive a monthly magazine and bulletins, which include current political and professional news related to counseling as well as announcements of professional development opportunities, job openings, and advertisements for professional products. Other benefits of membership include the ability to purchase professional liability insurance policies, reduced registration fees for the national convention and national workshops, and discounted prices for texts and media products published by ACA. Professional liability insurance is free to student members of ACA.

2-3 The Case of Juanita

Juanita recently received her master's degree in counseling and has taken her first full-time job as a counselor at an outpatient addictions treatment facility. Although her beginning salary was lower than she anticipated, Juanita is confident that with hard work and determination, she will get promotions and pay raises in the next few years. Immediately after beginning her job, Juanita paid a $200 fee to register with her state counselor licensure board so that she could complete her 2 years of supervised experience required to become licensed. She also decided, with a local licensed counselor, to meet weekly at $90 per meeting for supervision. During her 2 years in graduate school, Juanita had been a student member of ACA and two ACA divisions, the American Mental Health Counselors Association (AMHCA) and the International Association of Addiction and Offender Counselors (IAAOC). In addition, she had been a student member of her state counseling association and the state branch divisions of AMHCA and IAAOC. Juanita is also a member of the national counseling scholarship honorary society, Chi Sigma Iota, and is a National Certified Counselor (NCC). Both the organization and the certification board require annual dues or renewal payments. Juanita is on a tight budget and is thinking that she might not rejoin ACA and the two national divisions or her state counseling association and its two divisions.

- What will Juanita miss if she drops her ACA and national division memberships?
- What will Juanita miss if she drops her state counseling association and two state division memberships?
- How would you advise Juanita?

Discussion: Most importantly, if Juanita drops her ACA and national division memberships, she will miss the opportunity to stay informed of new developments in the counseling profession from a national perspective. She will no longer get ACA and division newsletters, announcements of national conferences, discounts for texts and conference registration fees, and bulletins on important matters of interest to counselors. Juanita will also miss opportunities for continued development of her professional identity as a counselor by not knowing what is going on in the counseling profession at the national level. She will no longer be able to purchase the high-quality professional liability insurance policy ACA makes available to its members.

If she drops her state association memberships, Juanita will miss the opportunity to stay informed of new developments in the counseling profession at the state level, where state licensure issues are addressed, and where funding for counseling positions and state laws affecting counselors are monitored. She will no longer get state counseling association and division newsletters, announcements of state conferences, discounts for conference registration fees, and bulletins on important matters of interest to counselors in her state. Juanita will also miss opportunities for continued development of her professional identity as a counselor by not knowing what is going on in the counseling profession at the state level and not being connected professionally to her state colleagues.

You might remind Juanita that she has spent considerable amounts of money to earn her master's degree in counseling. Not only has she paid for tuition and texts, but she has also lost income she would have earned by working while she was taking her graduate classes. Although all of these expenses may seem overwhelming to a new professional, if Juanita is to be successful in her career as a counselor, she will need to continue her professional association affiliations and maintain her counseling credentials. Counselors who isolate themselves from their professional peers seldom have successful careers. Professionals in all fields have similar expenses. These membership dues, certification renewal fees, and licensure board fees are part of the costs of being a professional.

ACA Divisions

Currently, ACA has 19 divisions. The American School Counselor Association (ASCA) and the American Mental Health Counselors Association (AMHCA) ended their division status with ACA in the past few years. ACA members, if they choose, may join one or more divisions in addition to their basic ACA membership. Division members receive scholarly journals from divisions that publish them, periodic newsletters, and, from time-to-time, information related to the divisions' special purposes.

There are two types of divisions within ACA, and all divisions are subgroups of ACA. One type of division has as members counselors who practice in various employment settings, such as colleges, rehabilitation agencies, or mental health centers. The other type of division has as members counselors who have special interests within counseling such as group counseling, multicultural counseling, or marriage and family counseling. Figure 2-2 provides a listing of the 19 ACA divisions with a description of their members.

FIGURE 2-2 The 19 divisions of the American Counseling Association

NAME OF ACA DIVISION	MEMBERS INCLUDE
Association for Adult Development and Aging (AADA)	Counselors who have a special interest in counseling issues of adults and those who specialize in gerontology
Association for Assessment and Research in Counseling (AARC)	Counselors who are interested in testing and research
Association for Child and Adolescent Counseling (ACAC)	Counselors who have a special interest in counseling children and adolescents in all settings
Association for Creativity in Counseling (ACC)	Counselors who have an interest in understanding creative approaches to counseling (such as art, music, or dance therapy)
American College Counseling Association (ACCA)	Counselors who work in college and university settings, including technical and community colleges
Association for Counselor Education and Supervision (ACES)	University professors who teach in counseling graduate programs, supervisors of counselors, and individuals who specialize in counseling supervision
The Association for Humanistic Counseling (AHC)	Counselors who are oriented toward humanistic principles
Association of Multicultural Counseling and Development (AMCD)	Counselors who have a special interest in multicultural issues in counseling
American Rehabilitation Counseling Association (ARCA)	Counselors who work in government and private vocational rehabilitation settings
Association for Spiritual, Ethical, and Religious Values in Counseling (ASERVIC)	Counselors who have special interests in spiritual, ethical, or religious values issues in counseling
Association for Specialists in Group Work (ASGW)	Counselors who have a special interest in group counseling
Counselors for Social Justice (CSJ)	Counselors who are interested in the eradication of oppressive systems of power and privilege
International Association for Resilience and Trauma Counseling (IARTC)	Counselors who are interested in trauma-informed practices and resilience
International Association of Addictions and Offender Counselors (IAAOC)	Counselors who work in addictions or prison settings
International Association of Marriage and Family Counselors (IAMFC)	Counselors who specialize in marriage and family counseling
Military and Government Counseling Association (MGCA)	Counselors who provide services to military personnel and government employees
National Career Development Association (NCDA)	Counselors who have a special interest in career counseling
National Employment Counseling Association (NECA)	Counselors who work in employment agency settings
Society for Sexual, Affectional, Intersex, and Gender Expansive Identities (SAIGE)	Counselors who have an interest in providing a nonthreatening counseling environment by aiding in the reduction of stereotypical thinking and homoprejudice

ACA State Branches

Most U.S. states, plus the District of Columbia, the Virgin Islands, Puerto Rico, the Philippines, and Europe, have ACA branches. Although these branches are chartered by ACA, they are separate, independent organizations. To join a state branch, you must contact that organization. Most branches offer discounted membership rates for graduate students. Telephone and address information for state branch associations can be obtained by contacting the ACA. State branches hold annual conventions, represent counselors from their states in legislative matters, publish periodic newsletters, and sometimes provide publications and workshops for members.

Most state ACA branches have state branch divisions that parallel the national ACA divisions. So, in addition to a state ACA association, often there will exist, for example, state divisions for school counselors, mental health counselors, and counselors interested in multicultural counseling issues. When you join your state ACA branch, you will also have the opportunity to join any divisions that exist in your state that interest you.

Many important professional issues, such as licensure and school counselor certification, can be addressed only at the state level. In addition, it is very important for counselors to know and interact with their colleagues throughout the state in which they are practicing (Darcy & Abed-Faghri, 2013). Therefore, throughout their professional careers, counselors should belong to their ACA state branch in addition to belonging to ACA and should provide leadership for the counseling profession (Barreto, 2012 & DeDiego et al., 2022; Fullen et al., 2020; Meany-Walen et al., 2013; Young et al., 2015).

Other Associations

In addition to ACA and its national divisions and state branches, counselors belong to many other national and international associations. These associations represent a variety of interests. Associations consist of multidisciplinary groups of mental health professionals, professionals interested in specializations within the mental health field, special political-purpose groups, and even associations that are in opposition to the professional and political interests of the ACA and counselors. It might be beneficial for counselors to join multidisciplinary professional associations to gain from cross-disciplinary interactions and fresh perspectives on mental health issues. Counselors may also want to purchase materials developed by other associations or attend workshops these other associations sponsor.

One association that was created because of ACA's success in state licensure for counselors is the American Association of State Counseling Boards (AASCB, 2023). This group has as its members state boards that license counselors and individuals who have been appointed to those boards or who administer them. The group meets annually to update members regarding developments in the area of counseling licensure.

Not all other mental health professional associations are supportive of counseling as a profession. The APA opposed counselors when counselors achieved recognition in state legislatures as an independent profession and currently opposes licensed counselors' efforts to pass legislation making it clear that counselors can diagnose and treat mental disorders, can test, and can be reimbursed by health insurance companies for their services.

Many associations focus on the certification of counselors and the accreditation of facilities that offer counseling services. Groups that have not already been mentioned in this chapter include the Commission on Accreditation of Rehabilitation Facilities (CARF International, 2018); the International Association of Counseling Services (IACS, 2018), which accredits university counseling centers; and The Joint Commission (2018), which accredits hospitals, home care organizations, laboratory services, and nursing care centers, among others. At the international level, the International Association for Counselling (https://www.iac-irtac.org), an international NGO with United Nations

consultative status, works to build collaborations among countries and promote the development of counseling in countries where the profession does not currently exist. Additionally, NBCC is active in initiatives to advance counseling and equitable access to mental health services worldwide.

CURRENT, LEGAL, AND POLITICAL ISSUES RELATED TO PROFESSIONAL IDENTITY

One of your tasks as a graduate student preparing to enter the counseling profession is to develop a strong identity as a counselor. Yet, as we have seen, the profession itself seems confused about its identity. Highlighting and discussing some of the profession's current identity problems may help you acknowledge and put these problems in perspective as you develop your own professional identity as a counselor.

Specialties Versus One United Profession

Probably the most significant problem that the counseling profession faces today is determining whether the profession will develop a strong identity as one united profession with a common philosophical foundation and knowledge base, or whether the specialties will emerge as the dominant force within counseling (Myers & Sweeney, 2001). If the specialties prevail, counseling has little chance of becoming a unified and societally recognized profession.

The problems associated with the specialties within the counseling profession were discussed 3 decades ago in the November/December 1995 issue of the *Journal of Counseling & Development* (Myers, 1995). In that issue of the primary professional journal of the counseling profession, authors discussed the following established and emerging counseling specialties: career counseling (Engels et al., 1995), college counseling (Dean & Meadows, 1995), school counseling (Paisley & Borders, 1995), marriage and family counseling (Smith et al., 1995), mental health counseling (Smith & Robinson, 1995), rehabilitation counseling (Leahy & Szymanski, 1995), addictions counseling (Page & Bailey, 1995), and sports counseling (Miller & Wooten, 1995).

A complex profession must include specialties because no one practitioner can be proficient in all areas of the profession (Buser et al., 2017; Tarvydas et al., 2017). Specialties are necessary to ensure that research, training, and improved practice occur in specialized areas of counseling, which have unique needs and issues. If members of subgroups within a profession believe that their specialties are significantly different from the general profession, however, then those specialty groups will attempt to establish themselves as separate professions (Etzioni, 1969). This phenomenon seems to continue to occur in counseling, particularly within the specialties of mental health counseling (Messina, 1999), marriage and family counseling, school counseling (Johnson, 2000), and career counseling. Both the American School Counseling Association and the Association for Mental Health Counseling were once ACA divisions, but they have disaffiliated themselves from the parent organization, which creates a burdensome expense for school counselors and mental health counselors who wish to affiliate with their specialty organization and support a unified counselor identity by joining ACA. Additionally, discomfort is created when counselors recognize that the specialty group members with whom they wish to affiliate may be working against the overall goal of establishing counseling as a strong, viable, and societally recognized mental health profession (Fox, 1994).

Our positions on this issue are that counseling is our primary profession and that counselors who are members of specialty groups should support the overall profession while paying attention to their special interests and needs within the counseling profession. When you enter the profession, you will have opportunities to voice your own position, and you will want to consider carefully the issues involved.

Organizational Structure of ACA

Historically, some specialty groups in counseling (school counselors, college counselors, career counselors, and counselor educators) came together to form the first professional association, which they named the American Personnel and Guidance Association (APGA) in 1952. The name was later changed to the American Association for Counseling and Development (AACD) and is now the American Counseling Association (ACA).

An unfortunate event occurred during the history of the association when, at some point, the divisions within ACA were incorporated as separate legal structures. This makes ACA appear to be a federation, or a group of groups, rather than one unified organization with subgroups of specialties. The separate incorporation of the divisions within ACA has led to many disagreements among association leaders as to whether ACA is a manager meant to serve the interests of the divisions or whether the divisions are subgroups of individuals whose primary interest is in being members of a unified professional association.

These disagreements led the American College Personnel Association (ACPA) to disaffiliate from ACA in 1992 and become a separate, autonomous professional association. The American School Counselor Association (ASCA) ended its ACA division status and become a separate association in 2018, and the American Mental Health Counselors Association (AMHCA) did the same in 2019. Two additional ACA divisions have seriously discussed disaffiliation, and have taken some actions that could lead to ending their relationships with ACA. Leaders and members of counseling associations have spent energy on the conflict rather than on the tasks of strengthening the counseling profession and ensuring that members' continuing professional development needs are met.

When ACPA left ACA, ACA immediately created a new division, the American College Counseling Association (ACCA), for members who worked in higher-education settings. In the future, other divisions might disaffiliate from ACA and establish themselves as separate, autonomous associations. If major divisions within ACA leave, ACA could create entities within ACA to replace them. In any eventuality, ACA members will be able to continue their affiliation with ACA and will have opportunities within ACA to affiliate with members who have their same specialty interests.

A basic aspect of human nature is that people tend to identify with the group they know best and with which they are most familiar. Members and leaders within the specialty subgroups in counseling have naturally tended to create well-organized entities that represent their specialized interests. It also is clear that counseling is at a critical stage of development (Collison, 2001; Lawson, 2016). Either the profession of counseling will find a way through its conflict and will emerge as a unified profession or it will fragment, and its various specialties will attempt to establish themselves as separate, independent professions. Bradley and Cox (2001) have suggested that perhaps a new organizational structure is needed that will allow the larger ACA to accommodate the differences within its divisions. Only time will reveal the outcome of the current struggles.

CACREP Accreditation of Specialties

Sweeney (1995) has pointed out that CACREP actually accredits master's-degree programs leading to specialty degrees in counseling, rather than accrediting general master's-degree programs in counseling. If a university that offers a master's-degree program in counseling wants to obtain CACREP accreditation, the program must declare that it prepares master's-level counselors for one or more of the following specialty areas: addictions; career; clinical mental health; clinical rehabilitation counseling; college counseling and student affairs; marriage, couple, and family counseling; rehabilitation counseling; or school counseling (CACREP, 2024). Even though CACREP accredits master's-degree programs leading to specialty degrees in counseling, a core curriculum that includes

basic instruction for all counselors is required for accreditation in any of the specialty areas. Bobby (2013) has explained how the CACREP standards have helped to unify the counseling profession.

This requirement that students specialize at the master's-degree level does cause some professional identity problems for graduate students. Although professors encourage graduate students to develop identities as counselors in general, these same graduate students are being told that they must choose and prepare for one specialty area within counseling. There are questions as well about what constitutes a particular specialty, such as clinical mental health counseling, and what makes it different from other specialties (Hershenson & Berger, 2001).

Varying State Licensure and Certification Requirements

As explained previously in this chapter, the states that license counselors vary significantly in their requirements. Most licensure statutes require 60 hours of graduate coursework and from 1 to 3 years of post–master's-degree supervision, and they include a variety of requirements that are unique to the state (ACA, 2023). State requirements for certification as a school counselor also vary considerably from jurisdiction to jurisdiction.

These differences in licensure and certification standards reflect a profession that has not been able to establish an agreed-upon set of standards and then to get those standards accepted by political entities throughout the United States. Because of this lack of uniformity, individual counselors who move from state to state often find that they must take additional coursework or even earn another master's degree to meet the new state's standards (Lawson et al., 2017). Established professions such as law, medicine, and even psychology have been able to establish licensing laws across the United States that are uniform and allow members of their professions to be licensed and to practice no matter where they live. Eventually, the licensing and certification laws for counselors will become more uniform, but currently, the lack of uniformity causes problems for counselors who relocate. Ohio's law that requires those seeking licensure as counselors to have graduated from a CACREP-accredited program beginning in 2018 (Bray & Lawson, 2013, 2016) may be the foundation for other states' passing similar laws. Leaders in the counseling profession and AASCB are working to resolve problems associated with licensure *portability* from state to state (Bohecker & Eissenstat, 2020; Mascari & Webber, 2013). A number of states have already adopted an interstate counseling compact, and many states are considering doing so. The compact allows counselors licensed in a compact state to practice in other compact states (ACA, 2023).

Legal and Political Issues

In addition to the professional-identity problems that currently exist for counselors, you must be familiar with a few legal and political issues. The successful resolution of these issues is critical to the continued movement of counseling to establish itself as a legitimate, societally recognized profession. Although many counselors are not comfortable with lobbying legislators (Schuster et al., 2021), political advocacy for the counseling profession often is necessary, as the issues discussed next illustrate.

CHALLENGES TO THE SCOPE OF PRACTICE OF COUNSELORS Counselors believe that they are qualified to perform many professional services for clients, based on their training and expertise. Two areas in which counselors have been challenged by psychologists and other mental health professionals are psychological testing and the diagnosis and treatment of mental and emotional disorders. Some psychologists and social workers claim that although they are adequately prepared to perform these professional services, counselors are not.

There is also an economic component to these issues. Counselors who test and diagnose and treat mental and emotional disorders in their practices often compete in the marketplace with psychologists and social workers. Many health insurance companies and health maintenance organizations require that mental health professionals be qualified to perform these tasks before they will reimburse their insured members for the services of these professionals. Therefore, there is a significant economic advantage for psychologists and social workers to claim that counselors are not qualified in these professional practice areas.

This debate has taken place in state legislatures in the form of arguments over the language in state statutes that license counselors for practice. In most states, counselors have been successful in inserting language into their state's licensing statutes that says they are competent and therefore qualified to test and to diagnose and treat mental and emotional disorders. There are some states, however, where these issues are still unresolved.

TESTING One of the core curriculum requirements for the preparation of counselors is testing, or assessment. All counseling master's-degree programs require at least one graduate course in testing principles that includes evaluating the validity and reliability of tests, selecting appropriate tests for clients, administering tests, and interpreting test results. Counselors have used tests in their practices since the profession was first established.

Of course, testing is a very broad area, and counselors might not be prepared at the master's-degree level to administer and interpret a number of psychological tests, such as individual intelligence tests or projective tests. Psychologists have claimed that counselors should not be allowed to test at all within their practices or should be allowed to use tests only in a very limited fashion. Counselors, on the other hand, believe that they have the basic education needed to test and that each counselor should determine the particular tests for which they are qualified by education or experience to administer and interpret.

Some state licensure statutes give counselors broad testing privileges, whereas others either are silent on whether testing is part of a counselor's practice or are very restrictive in specifying which tests a counselor may administer and interpret. In some states (Butler, 1998), counselors have been accused of practicing psychology without a license because of their testing activities. In states in which the testing practices of counselors are restricted, political activities are taking place to change the statutes to broaden the authority of counselors to test.

DIAGNOSIS AND TREATMENT OF MENTAL AND EMOTIONAL DISORDERS Counselors are taught to diagnose and treat mental disorders as part of their specialized training (Seligman, 1999). Ivey and Ivey (1999) have explained how counselors, with their wellness orientation, can utilize a developmental approach when using the *DSM* system. In most states, coursework is required in this area for counseling licensure. In addition, counselors are tested before they are licensed to determine whether they are proficient in the diagnosis and treatment of mental and emotional disorders. Despite the preparation of counselors in this area, it appears that the general public may also need to be better educated regarding the skills of counselors in the area of diagnosis and treatment of mental disorders (Fall et al., 2000).

The *Diagnostic and Statistical Manual of Mental Disorders* (*DSM*-5-TR), published by the American Psychiatric Association (2022), clearly states that many mental health professionals are engaged in the diagnosis and treatment of mental disorders and that the manual is meant to be a reference for all those professionals. The use of this standard manual is taught to counselors who wish to be licensed.

As with the testing issue, in many states the scope of practice of licensed counselors includes the diagnosis and treatment of mental disorders. In states in which the authority of counselors to

diagnose and treat mental disorders is being challenged, efforts are being made to change the language of the licensure statutes to state clearly that counselors are qualified to perform this professional activity (Kassirer et al., 2013).

JOB CLASSIFICATIONS FOR COUNSELORS Positions for counseling professionals exist in many state and federal agencies. In some of these agencies, however, no job classification categories have been established for counselors. As a result, when counselors are hired, their job titles might be *psychologist I, social worker I*, or *mental health technician I*. Although being hired into these positions gives counselors jobs and entry into agencies, counselors may be discriminated against later because promotions might require a license in psychology or social work or a graduate degree in one of those fields.

2-4 The Case of Elijah

Elijah has been working for 7 years as a counselor in a public mental health center that serves underprivileged young men and women who have been released from prison and are seeking jobs and trying to reintegrate into society. He was hired in the position as soon as he graduated from his counseling master's-degree program, and he got his state license as a counselor 4 years ago. Elijah finds the work he does with his clients professionally rewarding and believes he could stay with this state agency his entire career despite some frustrations with state rules and regulations that he finds annoying. The agency Elijah works for is a unit of a state agency entitled "Human Services." A Secretary of Human Services appointed by the state governor is the top-level administrator for the agency. The current Secretary is a former state senator who has a background in banking. There are a number of Assistant Secretaries of Human Services who are state employee bureaucrats who report to the Secretary. The agency Elijah works for is under the authority of the Assistant Secretary for Social Welfare, who has a Master's of Social Work (MSW) degree, is licensed by the state as a social worker, and has been working for the state agency for 30 years.

Elijah and many other licensed counselors who work for the state agency were hired with a "Social Worker I" job title, which required a minimum of a bachelor's degree in a "social services field" because there were no job titles for counselors at the time Elijah and the other counselors were hired. Five levels of social worker job titles exist that are promotions that offer employees higher pay ranges for continuing to do the same job. Elijah was recently promoted to Social Worker II, which required a minimum of 5 years of successful experience as a Social Worker I with positive annual evaluations and a master's degree in a social services field. Elijah knows from reading the personnel manual that to be promoted to a Social Worker III position, individuals must have a minimum of 8 years of successful experience and hold an MSW. Elijah is concerned that even after he has a minimum of 8 years of successful experience with the agency, he will never get promoted to Social Worker III because he does not have an MSW. Elijah has discussed his concern with his immediate supervisor, who holds an MSW, but his supervisor's response was that perhaps Elijah should consider going back to the university to earn a master's degree in social work.

- What agency changes are necessary for Elijah and other counselors with counseling master's degrees to be eligible for future promotions?
- What are the options available to Elijah and other master's-level counselors in the agency regarding their future promotion opportunities?

Discussion: In order to for Elijah and other master's-level counselors to secure advanced career opportunities in their agency, they must either get the agency to create a series of job titles for counselors that have counselor-credential requirements or get the agency to change the "Social Worker" job title series requirements so that they can qualify for the additional levels of "Social Worker" job titles. From a professional identity perspective and for future counselors hired by the agency, it would be preferable for Elijah and the other counselors to advocate for a new job title series to be established that has the word "counselor" in the title.

Elijah and his fellow counselors could advocate for themselves and the profession by making proposals internal to the agency to get a counselor job series established that would be equivalent to the social worker series. They could do this by writing proposals to administrators, beginning at the bottom level and working up to the Secretary of Human Services. However, it is possible that lower-level administrators who hold MSW degrees may block or ignore such proposals.

If Elijah and the other counselors are not able to work internally to get a counselor job series established, then they might work with their state counseling association to bring political pressure on the agency secretary to make the changes. If that approach does not work, then the state counseling association might get a legislator to introduce legislation that would establish a counselor job series in the agency.

As a result of this problem, efforts are being made to have job titles established specifically for counselors. Convincing a state or federal agency that job titles or classifications should be changed is not an easy task. Bureaucracies are very slow to change. Nonetheless, professional associations and leaders within the counseling profession must address this problem if counselors are to be fully accepted as professionals within these agencies.

In Louisiana, a series of job categories for counselors who work in the state civil service system was created in 2003 (C. Gagnon, personal communication, September 10, 2003). Until that time, counselors were hired in job categories that limited their ability to receive promotions. Creation of these new counselor job categories (which are equivalent to those that existed already for social workers) was completed in a unique manner because of the political and legal environments in that state. The success in this endeavor has had a significant, positive impact on all counselors currently working in that state and counselors who will be hired there in the future. NBCC reported that in July 2014, a U.S. Senate subcommittee went on record as urging the creation of an occupation series for mental health counselors (NBCC, 2023).

THIRD-PARTY REIMBURSEMENT It is essential for counselors in private practice that their clients be able to access their counselors' mental health services as part of the clients' health care plan (Palmo, 1999). If clients are participating in health maintenance organizations (HMOs) or preferred provider organizations (PPOs), then counselors must be on the list of eligible providers for the clients. If clients are part of an indemnity health care plan, then counselors must be acknowledged as qualified mental health care providers so that clients can be reimbursed for counselor services.

HMOs, PPOs, and health insurance companies can voluntarily acknowledge counselors as qualified mental health care providers, and most do. However, these health care organizations have a legal right to refuse to give their clients access to counselors unless there is legislation to the contrary. This type of legislation, called *freedom of choice legislation*, requires health care providers to give access

to licensed counselors for mental health care if the providers give access to other mental health care providers such as psychologists or social workers. Many states have passed freedom of choice legislation. In states that have not, counselors are actively trying to get freedom of choice legislation passed.

ACA and NBCC are also active in seeking national legislative and regulatory changes that would allow Medicaid recipients, military personnel, and federal employees to receive mental health services from counselors as part of their health benefits. An important goal is for all Americans to have access to mental health services from counselors, not just from psychiatrists, psychologists, or social workers.

Counselors have been successful in being recognized as providers for individuals who are on active duty in the military and their dependents through TRICARE (ACA, 2023; NBCC, 2023), and as providers of mental health services to Medicare recipients.

IDENTITY AND PROFESSIONALISM

Counseling and Other Mental Health Professions

As we noted at the beginning of this chapter, counselors are members of a relatively new professional group, and counselors are often asked what they do. A weak answer would be that counselors do things similar to what psychologists do. A strong answer would be to describe the process of counseling, including its philosophical foundations, the services that counselors provide, and the components of training programs that prepare master's-level counselors. A counselor or prospective counselor with a strong professional identity will be able to describe the process of counseling without reference to other mental health professions.

The material provided in this chapter should allow you to state clearly what a counselor does. However, a further challenge for counselors is to be able to describe objectively, accurately, and dispassionately the similarities and differences between counselors and other mental health professionals. Listeners should take away from a conversation with a counselor the conclusion that counselors have a clear and unique identity within the field of mental health and that the counselor with whom they have been talking takes pride in her or his chosen profession.

How are counselors similar to other mental health professionals, including psychiatrists, psychologists, and social workers? They are all prepared at the master's-degree level or higher, and they all provide mental health services for clients. All mental health professionals follow a similar process of assessing a person's mental health needs, developing a plan for assisting the person, and then providing mental health services consistent with the assessment and plan for treatment. All mental health professionals counsel clients in some fashion. Mental health professionals of all types can be found in agencies and institutions where mental health treatment is offered to individuals.

So, how are counselors different from other mental health professionals? The primary difference between counselors and other mental health professionals is that counselors espouse the wellness model of mental health instead of the illness or medical model. The wellness model of mental health emphasizes helping people maximize their potential rather than curing their illness. Counselors believe that human growth and development stages and processes make life's problems normal and natural rather than pathological. Counselors emphasize prevention and early intervention rather than remediation. Empowerment of clients is another goal of counselors. The training of counselors focuses on teaching counseling skills rather than skills in physical health care, psychopathology, assessment, or social service linkages.

Definitions of the mental health professions, including counseling, have been published by professional associations. The definition of *counselor* is from the American Counseling Association (2014); the definition of *social worker* is from the Council on Social Work Education (2014); the

definition of *psychologist* is from the American Psychological Association (2014); and the definition of *psychiatrist* is from the American Psychiatric Association, as provided by MedicineNet (2008). These definitions provide insight into how professional organizations view themselves. The *counselor* definition focuses on the counseling process that was derived from mental health, psychological, and human development principles. The *social worker* definition emphasizes the goals of promoting social justice, human rights, and quality of life as well as eliminating poverty. The *psychologist* definition states the requirements that psychologists hold doctoral degrees and that psychologists focus on understanding human behavior. The *psychiatrist* definition begins by saying that psychiatry is a specialty for physicians who focus on mental illnesses. Figure 2-3 summarizes the differences in preparation program requirements for the major mental health professions of counseling, social work, psychology, and psychiatry. This figure was developed using information provided by the organizations that accredit these four mental health professions: for counseling, the Council on Accreditation of Counseling and Related Educational Programs (CACREP); for social work, the Council on Social Work Education (CSWE); for psychology, the American Psychological Association Commission on Accreditation; and for psychiatry, the Liaison Committee on Medical Education (LCME) and the American Board of Psychiatry and Neurology (ABPN). This figure illustrates the differences in the amount of post–bachelor's-degree study required (ranging from 2 years of graduate study for counseling and social work and 3 years of graduate study for psychology, to 7 years of graduate study for psychiatry). The content of the coursework requirements demonstrates that the counseling curriculum has a primary focus on counseling; the social work curriculum emphasizes social issues; the psychology curriculum has a heavy emphasis on evaluation and research; and the psychiatry curriculum prepares physicians who specialize in behavior disorders and neuroscience. An examination of the required courses within accredited counseling, social work, psychology, and psychiatry programs would further demonstrate the different emphases of the preparation programs for these mental health professions.

The similarities to and differences from other mental health professionals, as previously summarized, are easy to articulate. It is vital that counselors learn to state both the traits we share in common with our other mental health colleagues and the differences that make counselors unique as mental health care providers (Sweeney, 2001).

In addition to understanding the differences and similarities, it is important to keep in mind that you will also have working relationships with other mental health professionals when you enter practice as a counselor. Psychologists often contribute articles to counseling professional journals (Weinrach et al., 1998, 2001). In addition, many counselors work closely with other mental health professionals on a day-to-day basis—for example, as members of multidisciplinary treatment teams in hospitals or as members of school teams that provide services to students with special needs (Goodman-Scott & Boulden, 2019; Walsh & Gibson, 2019). Increasingly, collaboration with other mental health professionals is regarded as best practice, so it is vital for counselors to understand the scope of practice of these other professionals. After studying the effectiveness of mental health counselors in an integrated primary and behavioral health care environment, Ulupinar et al. (2021) concluded that counselors perform well in integrated systems of care.

In a previous section we discussed some conflicts that currently exist between counselors and certain other mental health professional groups. Despite the likelihood that you and your colleagues will disagree about professional issues, in the practice environment these differences are always set aside in the interest of providing clients with the best possible mental health services (ACA, 2014, §D.1.c.). You have an ethical obligation to respect approaches to mental health treatment that are different from your own and to acknowledge the expertise of other professional groups and be respectful of their practices (§D.1.a.).

FIGURE 2-3 Comparison of preparation program requirements for the mental health professions

COMPARISON OF PREPARATION PROGRAM REQUIREMENTS FOR THE MENTAL HEALTH PROFESSIONS			
Profession	**Graduate Education Required**	**Summary of Required Courses**	**Practicum Hours and Required Supervised Field Experience**
Counseling	60 hours required; Master's degree	Graduate coursework required in (a) professional counseling orientation and ethical practice, (b) social and cultural identities and experiences, (c) lifespan development, (d) career development, (e) counseling practice and relationships, (f) group; and specialty area (addictions; career; clinical mental health; clinical rehabilitation counseling; college counseling and student affairs; marriage, couple, and family counseling; rehabilitation counseling; or school counseling).	A 100-hour practicum and a 600-hour internship required
Social Work	60 hours required; Master's degree	Coursework required in professional social worker identity; ethical principles; critical thinking; diversity and difference; advancing human rights and social and economic justice; research-informed practice and practice-informed research; human behavior and the social environment; policy practice; contexts that shape practice; and engaging, assessing, intervening, and evaluating individuals, families, groups, organizations, and communities.	A total of 900 hours of field experience required
Psychology	3 full-time years of graduate study required for doctoral degree	Coursework required in biological aspects of behavior; cognitive and affective aspects of behavior; social aspects of behavior; history and systems of psychology; psychological measurement; research methodology; techniques of data analysis; individual differences in behavior; human development; dysfunctional behavior or psychopathology; professional standards and ethics; theories and methods of assessment and diagnosis; effective intervention; consultation and supervision; evaluating the efficacy of interventions; cultural and individual diversity; attitudes essential for life-long learning, scholarly inquiry, and professional problem solving.	One full-time year of residency required
Psychiatry	130 weeks required for medical degree (usually 4 years)	Medical degree requires coursework in anatomy; biochemistry; genetics; physiology; microbiology and immunology; pathology; pharmacology and therapeutics; preventive medicine; the scientific method; accurate observation of biomedical phenomena; critical analysis of data; organ systems; preventive, acute, chronic, continuing, rehabilitative, and end-of-life care; clinical	36-month residency in psychiatry required

FIGURE 2-3 (*continued*)

COMPARISON OF PREPARATION PROGRAM REQUIREMENTS FOR THE MENTAL HEALTH PROFESSIONS			
Profession	**Graduate Education Required**	**Summary of Required Courses**	**Practicum Hours and Required Supervised Field Experience**
		experiences in primary care, family medicine, internal medicine, obstetrics and gynecology, pediatrics, psychiatry, and surgery in outpatient and inpatient settings; multidisciplinary content areas, such as emergency medicine and geriatrics; disciplines that support general medical practice, such as diagnostic imaging and clinical pathology; clinical and translational research, including how such research is conducted, evaluated, explained to patients, and applied to patient care; communication skills as they relate to physician responsibilities, including communication with patients, families, colleagues, and other health professionals; addressing the medical consequences of common societal problems—for example, providing instruction in the diagnosis, prevention, appropriate reporting of, and treatment of violence; the manner in which people of diverse cultures and belief systems perceive health and illness and respond to various symptoms, diseases, and treatments; gender and cultural biases; medical ethics and human values. The psychiatry residency must prepare physicians in the psychiatric areas of development in the life cycle; behavioral and social sciences; epidemiology and public policy; diagnostic procedures; clinical aspects of psychiatric disorders; treatment of psychiatric disorders; special topics (suicide; dangerousness; seclusion/restraint; risk management; child abuse, sexual abuse, and domestic violence; psychiatric consultation; professionalism/ethics; other). In addition, the psychiatry residency must prepare physicians in the neurologic areas of development through the life cycle; basic neurosciences; diagnostic procedures; clinical aspect of neuropsychiatric disorders; treatment of neuropsychiatric disorders; diagnostic and clinical evaluation of neurologic disorders/syndromes; and management and treatment of neurologic disorders.	

Pride in the Counseling Profession

In Chapter 1, we defined professionalism as an internal motivation to perform at the level of *best practices* that represent the ideals of the profession, to enhance its image, and to promote its development. A crucial component of professionalism is having a sense of pride in one's chosen profession. Counselors who have strong professional identities are proud to be members of their profession. By understanding and appreciating the counseling profession's history, philosophical foundations, services that are offered to the public, training program contents, and similarities to and differences from other mental health professions, counselors will be satisfied with their chosen profession and communicate this pride to those with whom they come into contact.

It is important for members of a profession to be self-critical and to seek advancements in knowledge and improvements within the profession. At the same time, members of a profession must be comfortable with the career choice they have made in order to develop a professional identity that serves them and the profession well.

Counselors with strong professional identities express pride in their profession by defending the profession when inaccurate statements are made concerning the profession itself or members of the profession. To represent the profession whole-heartedly to the public, counselors must be thoroughly familiar with the information presented in this chapter. Individual counselors who provide quality counseling services are responsible for the positive reputation that the profession enjoys. Similarly, individual counselors who can articulate the strengths of the profession are responsible for informing the public about the unique contributions to society that members of the counseling profession are making.

Counseling Around the World

Although counseling is a profession that was established and developed in the United States, there is evidence that the counseling profession, often in a form like that which exists in the United States, is being established throughout the world. In other words, today's counseling profession is trending toward globalization (Guth et al., 2014; Lorelle et al., 2012; Shannonhouse & Myers, 2014).

Despite cultural differences among countries, there appears to be an interest worldwide in having counseling services available to citizens. It appears that counseling imported from the United States is being adapted to the cultures of the countries where counseling is being established.

Different countries have a variety of political environments that are affecting the ways in which counseling is developing. For example, in many European countries, counseling is not accepted as a field of study in universities, so counselors are being prepared in private schools. In some African countries, counseling was established to help citizens cope with the AIDS epidemic (Luke et al., 2016). In many Asian countries, counseling is closely following the model of the profession in the United States, despite the vast cultural differences between Asians and Americans. As will be further discussed in Chapter 3, concerns have been raised regarding the need for cultural humility when working with mental health providers in other countries to prevent colonization.

A book published by the American Counseling Association, *Counseling Around the World: An International Handbook*, edited by Hohenshil, Amundson, and Niles (2017), summarizes how the profession of counseling is developing in 40 countries located on six continents. In addition, the leading journal in the counseling profession, the *Journal of Counseling and Development*, has published a series of articles summarizing the beginnings of the counseling profession in a multitude of

countries including, but not limited to, Australia (Schofield, 2013); Ecuador (Smith & Valarezo, 2013); Hong Kong (Yuen et al., 2014); Italy (Remley et al., 2010); Russia (Currie et al., 2012); South Korea (Lee et al., 2012); Switzerland (Thomas & Henning, 2012); Taiwan (Guo et al., 2013); and Uganda (Senyonyi & Ochieng, 2012). In addition, the development of counseling in other countries has been documented in China (Shi et al., 2014); England (McCarthy, 2014); Ireland (McCarthy, 2011, 2014); Mexico (Machorro & Suck, 2014); Scotland (Griesbach et al., 2008); Sierra Leone (Akinsulure-Smith & Conteh, 2018); Southeast Asia (Thakore-Dunlap & Van Velsor, 2014); and Venezuela (Vera, 2011).

CACREP has established a registry that includes counselor preparation programs that are different from those in the United States but meet CACREP standards (CACREP, 2024). The registry is known as the International Registry of Counsellor Education Programs (IRCEP) and can be found at www.ircep.org/directory.

Summary and Key Points

The aim of this chapter is to help you clarify and strengthen your professional identity as a counselor. We hope that after reading the material in this chapter, you will have a clear sense of your professional identity and be able to do the following:

- Explain the philosophy that underlies the activities of your professional group
- Describe the services your profession renders to the public
- Describe the components of your profession's preparation programs
- Articulate the similarities and differences between members of your profession and other, similar professional groups
- Communicate your pride in being a member of the counseling profession

We began the chapter with a discussion of the philosophy underlying the counseling profession. This philosophy, which makes the counseling profession unique and distinguishes counseling from other mental health professions, includes the following four components: the wellness model of mental health, a developmental perspective, a strong preference for prevention of and early intervention into problems, and a goal of empowerment for clients. We discussed the impact of COVID-19 on the profession of counseling. Virtual counseling was unusual before COVID-19 but now is the norm.

Other aspects of professional identity presented in this chapter include counselor preparation programs; credentialing; professional associations for counselors; and current professional, political, and legal issues. A brief history of the counseling profession provided a foundation for a careful look at its present status as a relatively new profession. Following are some of the key points of this chapter:

- Vital professional tasks for counselor trainees are to develop a strong professional identity and to be able to articulate that identity to others.
- Counselors espouse the wellness model of mental health as their perspective for helping people. Other mental health professions have their roots in the tradition of the medical model. Although the differences between the two traditions are clear, in actual practice many other mental health practitioners use the wellness model, and counselors are educated to use the medical model of diagnosing mental disorders.
- Counselors take a developmental perspective to understanding people's personal and emotional issues and problems. From this perspective, problems that other mental health professionals might view as pathological are seen by counselors as natural and normal.
- Counselors believe that prevention of and early intervention into mental health problems are far superior to remediation of mental health problems.
- The goal of counseling is to empower clients so that they will be able, in the future, to resolve their problems independently of counselor assistance.

- For counselors, mental health counseling is the primary service they provide. By contrast, other mental health professionals provide counseling as a secondary or ancillary service.
- Counselors are considered to be professionals after they have completed their master's degree.
- Preparation programs emphasize the development of counseling skills and are generally located in colleges of education within universities.
- Credentialing in counseling is complicated and can be confusing. It is important that you can distinguish among state licensure, state certification, national voluntary certification, and program accreditation. It is also important that you plan your program of studies carefully so that you will qualify for the credentials you will need to practice effectively.
- The counseling profession emerged out of a convergence of several disparate forces, including counseling psychology, school counseling, and vocational rehabilitation counseling.
- The field of counseling meets the criteria generally accepted as constituting a mature profession and is making rapid progress toward becoming a societally recognized profession.
- The primary professional association for counselors is the ACA. This organization has various divisions and state branches that serve the particular needs and interests of its members. You should join ACA now, if you have not already done so.
- Currently, the counseling profession is struggling with several professional-identity problems. One issue currently being debated is whether counseling will develop as a united profession or as separate specialties. Another issue that is being decided is the organizational structure of ACA. A third problem is that requirements for becoming licensed as a counselor vary from state to state.
- Contemporary legal and professional issues include challenges to the scope and practice of counselors, particularly in the areas of testing, diagnosis, and treatment of mental disorders; job classifications for counselors; and third-party reimbursement for counseling services.
- Counselors have an ethical obligation to maintain positive working relationships with other mental health professionals and to respect and understand their differing approaches.
- Having a sense of pride in being a member of the counseling profession is essential both for your own internal satisfaction with your chosen career and for the continued progress of counseling toward being a societally recognized profession.
- The profession of counseling is developing in countries around the world. Each country is adapting the profession to fit its own political climate and cultural heritage.